

LEARNER GUIDE

Unit Standard Title:	Apply knowledge of HIV/AIDS to a specific business sector and a workplace
Unit Standard No:	114941
Unit Standard Credits:	4
NQF Level:	3

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This material has been compiled based on the **UNIT STANDARD: 114941 Apply knowledge of HIV/AIDS to a specific business sector and a workplace**

The program has been accredited by SASSETA in the Republic of South Africa.

Material compiled by:

Training Dynamics Africa

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1. Introduction

This learning program is part of a complete qualification. For more details concerning the complete qualification please contact your service provider or alternatively the office of the SASSETA

2. Purpose of this learning program

This unit standard provides a broad introduction to HIV/AIDS in the workplace. It introduces knowledge about HIV/AIDS that will enable learners to be informed and caring workers in an industry and managers to develop appropriate policies to deal with the pandemic. The focus is knowledge, skills, values and attitudes in relation to the learner and management with a view to creating a caring environment in the workplace and the community.

A manager in the context of this Unit Standard is the manager of a small business and first line managers of business units, team leaders and supervisors in medium and large organizations. The term business unit implies a small business, cost centre, section or department.

The Unit Standard was developed as a means of contributing to change in leadership focus and empowering managers to look at people issues.

The qualifying learner is capable of:

Explaining HIV/AIDS.

- Describing what behaviour is safe and what behaviour carries the risk of HIV/AIDS transmission.
- Interpreting data and trends on HIV/AIDS in order to explain the potential impact on a community, an organisation and a business sector.
- Investigating the guidelines and assistance that are available to support workers affected by HIV/AIDS.
- Explaining the implications of the HIV/AIDS pandemic for the community, the economy, an organization and a specific workplace.

2.1 Target group

This program is compiled for the following **target group**:

- Security members
- South African Defense Force members
- South African Police Force members
- Correctional Services
- Individuals who wishes to complete the NQF level 3 National Certificate in security practices.

3. Standards and qualifications

It is important that you as the learner understand and realize that this training program is not presented in isolation, but that it was developed based upon nationally recognized standards known as unit standards.

Unit standards are the “building blocks” of qualifications. All qualifications are plotted on the National Qualifications Framework (NQF).

REMEMBER.....

Unit standards comprises of outcomes. An outcome is a statement that describes the required competency that must be demonstrated by the learner on successful completion of a training intervention.

4. Assessments

In order to assess whether a learner can actually demonstrate the desired outcomes, assessment criteria are included in the unit standard. Each outcome has its own set of assessment criteria.

REMEMBER.....

The assessment criteria describe the evidence that is needed that will show that the learner has demonstrated the outcome correctly.

Kindly refer to the unit standard attached hereto for the assessment criteria.

It is of utmost importance that the learner fully understands the assessment criteria as listed in the unit standard, as it is the only way in which the learner will know what he will be assessed against.

The **final or summative assessment** is the most important aspect of this training program. It is during this process that the learner will be declared competent or not yet competent.

The learner will know exactly how he will be assessed, and when and where he will be assessed. All of these details must be obtained from the training provider where the learner enrolled for his program.

Each of the modules consists of learner outcomes to be achieved during training and development interventions, work related experience or at your own time. The learner outcomes will enable you to achieve the required level of confidence and competence in order to undergo the summative (final evaluations) assessment.

5. Range statements

Also included in the unit standard are the range statements in support of the assessment criteria.



The range statements indicate detailed requirements of the assessment criteria

6. The learning guide

The learner guide is included in this material under various learning units. The learner guide has been designed in such a manner that the learner is guided in a logical way through the learning material and requirements of the unit standard.

On completion of this skill program, the learner will be assessed against the assessment criteria as stipulated in the unit standard.

The learner needs to assess his own knowledge and skill throughout the training process by completing the learner workbook. With the completion of the workbook you start collecting evidence to proof your competence. The learner workbook will be assessed (FORMATIVE ASSESSMENT)



The learner guide will remain the property of the learner once the LEARNING PROGRAM has been completed.



- The best results will be obtained if you start with Study Unit 1 in the Learner Guide, and work your way through it, study unit by study unit.
- All learning outcomes are vital and must be studied (and exercised for all practical requirements) thoroughly to ensure that enough evidence for all specific outcomes is generated.

7. Learning assumed to be in place

It is assumed that a learner which registers for this program have completed a program in communication at NQF Level 3 or equivalent.

It is imperative to understand that this program are only one of the building blocks to complete the National Certificate: General Security Practices, NQF Level 3, 124 Credits

	ID	UNIT STANDARD TITLE	LEVEL	CREDITS
Core	244176	Use security equipment	Level 2	2
Core	114941	Apply knowledge of HIV/AIDS to a specific business sector and a workplace	Level 3	4
Core	244184	Apply legal aspects in a security environment	Level 3	8
Core	244177	Conduct a security patrol in an area of responsibility	Level 3	7
Core	117705	Demonstrate knowledge of the Firearms Control Act 2000 (Act No 60 of 2000) applicable to possessing a firearm	Level 3	3
Core	246694	Explain the requirements for becoming a security service provider	Level 3	4
Core	244182	Give evidence in court	Level 3	4
Core	244179	Handle complaints and problems	Level 3	6
Core	244181	Perform hand over and take over responsibilities	Level 3	2
Core	244189	Conduct access and egress control	Level 4	7
Core	242825	Conduct evacuations and emergency drills	Level 4	4
Core	11505	Identify, handle and defuse security related conflict	Level 4	12
Fundamental	119472	Accommodate audience and context needs in oral/signed communication	Level 3	5
Fundamental	9010	Demonstrate an understanding of the use of different number bases and measurement units and an awareness of error in the context of relevant calculations	Level 3	2
Fundamental	9013	Describe, apply, analyse and calculate shape and motion in 2- and 3-dimensional space in different contexts	Level 3	4
Fundamental	119457	Interpret and use information from texts	Level 3	5
Fundamental	9012	Investigate life and work related problems using data and probabilities	Level 3	5
Fundamental	119467	Use language and communication in occupational learning programmes	Level 3	5
Fundamental	7456	Use mathematics to investigate and monitor the financial aspects of personal, business and national issues	Level 3	5
Fundamental	119465	Write/present/sign texts for a range of communicative contexts	Level 3	5
Elective	117889	Demonstrate an understanding of Victim Empowerment	Level 2	6
Elective	12484	Perform basic fire fighting	Level 2	4
Elective	13912	Apply knowledge of self and team in order to develop a plan to enhance team performance	Level 3	5
Elective	244178	Apply neighbourhood watch principles	Level 3	5
Elective	113852	Apply occupational health, safety and environmental principles	Level 3	10
Elective	243188	Care for a service dog	Level 3	15
Elective	116534	Carry out basic first aid treatment in the workplace	Level 3	2

UNIT STANDARD: 114941 Apply knowledge of HIV/AIDS to a specific business sector and a workplace

Elective	113909	Coach a team member in order to enhance individual performance in work environment	Level 3	5
Elective	114942	Describe how to manage reactions arising from a traumatic event	Level 3	2
Elective	243190	Handle a trained service dog to deter crime	Level 3	8
Elective	119649	Handle and use a handgun	Level 3	2
Elective	119651	Handle and use a manually operated rifle or carbine	Level 3	2
Elective	119650	Handle and use a self-loading rifle or carbine	Level 3	2
Elective	119652	Handle and use a shotgun	Level 3	2
Elective	114979	Operate a computer workstation in a business environment	Level 3	2
Elective	244335	Conduct security at an event	Level 4	5
Elective	252174	Demonstrate an understanding of security measures within a port security environment	Level 4	4
Elective	246693	Explain the requirements for safeguarding a national key point	Level 4	4
Elective	123515	Handle and use a handgun for business purposes	Level 4	3
Elective	123519	Handle and use a manually operated rifle or carbine for business purposes	Level 4	3
Elective	123511	Handle and use a self loading rifle or carbine for business	Level 4	3
Elective	123514	Handle and use a shotgun for business purposes	Level 4	3
Elective	11985	Inspect, drive and maintain an official vehicle	Level 4	3
Elective	11513	Operate effectively within a specified control room environment	Level 4	15

8. Preparation of summative assessment

After you have worked through the **LEARNER GUIDE** and you are satisfied that all theoretical and practical requirements/ evidence, as stipulated by the UNIT STANDARD, can be met, you need to make your own necessary arrangements with the assessor and/ or facilitator. The training establishment will then arrange a suitable date for you to attend your pre-assessment meeting and then arrange for a suitable date to complete the final assessment.

8.1 RPL assessment

The assessment of RPL learners will be conducted in the same way as for those of new learners. The assessment pack is exactly the same and will therefore be used for new learners as well as RPL Learners. It must however be noted that learners who are applying for RPL must provide proof of previous learning and subject related experience prior to the assessment.

This proof or evidence can be in the format of certified copies (certificates) of previous learning programs that have been attended.



All the evidence will be assessed and authenticated before a learner will be allowed to enroll for a RPL program.

9. Roles and responsibilities**9.1 Facilitator/instructor**

- Prepare to facilitate
- Transfer of knowledge and skills

9.2 The role of the assessor

- Plan for the assessment
- Prepare the learner for the assessment

9.3 The role of the Learner

- Prepare for assessment
- Accept the responsibility of learning
- Accept the responsibility to produce/deliver evidence

STUDY UNIT 1

Understanding HIV

LEARNING OUTCOMES

On completion of this study unit the student will....



- ♦ Define the term “HIV”.
- ♦ Explain HIV and AIDS.
- ♦ Demonstrate knowledge on the effect of HIV on the body by explaining the effects.
- ♦ Explain the immune system and the functions of it.
- ♦ Explain the effects of HIV on the immune system.
- ♦ Explain what is meant by a window period.
- ♦ Explain the symptoms of HIV.
- ♦ Explain the stages of HIV infection.
- ♦ Understand and explain Opportunistic infections
- ♦ Explain ways on how an individual can get aids.

1. What is HIV?

DEFINITION

HIV stands for human immunodeficiency virus. When the HI Virus enters the body, it attacks and slowly destroys the immune system (i.e. the body's defence system against infections and diseases).

As a result, a person who has the HI Virus will find it harder and harder to resist and fight infections and diseases which lead to serious illnesses, and often death.

IMPORTANT

What's important to remember is that it may take ten years, or even longer, for the HI Virus to totally destroy the immune system. That's why a person who is infected may look and feel well for a long time before they have any symptoms of HIV or become seriously ill. Also remember that a person with HIV does not necessarily have AIDS.

2. What is AIDS?

DEFINITION

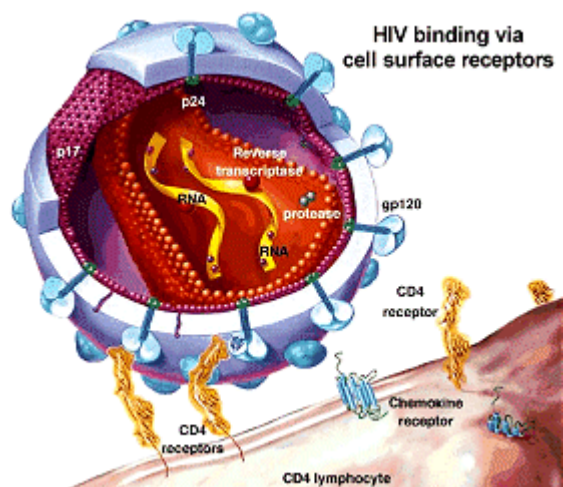
AIDS, which stands for acquired immune efficiency syndrome, is the final stage of HIV. It's not a single disease but rather a combination of different illnesses caused by the immune system's total breakdown and inability to protect the body against opportunistic infections e.g. oral thrush, Tuberculosis (TB), etc.

3. How does HIV affect the body?

Normally, the human immune system is the body's protection against bacteria's, viruses, etc.; it is like a coat of armor. When HIV enters the body, it starts poking holes in the armor. Eventually, the armor becomes very weak and unable to protect the body. Once the armor is very weak or is gone, the person is said to have AIDS. An AIDS diagnosis is generally made when either the body's protective T-cells drop below a certain level, or the HIV-positive individual begins to experience **opportunistic infections**.

DEFINITION

An opportunistic infection is an infection that would not normally affect an otherwise healthy person. Oftentimes, it's these infections that are the cause of illness or death in HIV-positive individuals - not the virus itself. If people do not get any treatment for HIV disease, it takes an average of 8-10 years to progress from HIV to AIDS.



4. The Immune System

The immune system, which is made up of special cells, proteins, tissues, and organs, defends people against germs and microorganisms every day. In most cases, the immune system does a great job of keeping people healthy and preventing infections. But sometimes problems with the immune system can lead to illness and infection.

4.1 What the Immune System Does

The immune system is the body's defense against infectious organisms and other invaders. Through a series of steps called the **immune response**, the immune system attacks organisms and substances that invade our systems and cause disease. The immune system is made up of a network of cells, tissues, and organs that work together to protect the body.

The cells that are part of this defense system are white blood cells, or **leukocytes**. They come in two basic types, which combine to seek out and destroy the organisms or substances that cause disease.

Leukocytes are produced or stored in many locations throughout the body, including the thymus, spleen, and bone marrow. For this reason, they are called the **lymphoid** organs. There are also clumps of lymphoid tissue throughout the body, primarily in the form of lymph nodes, that house the leukocytes.

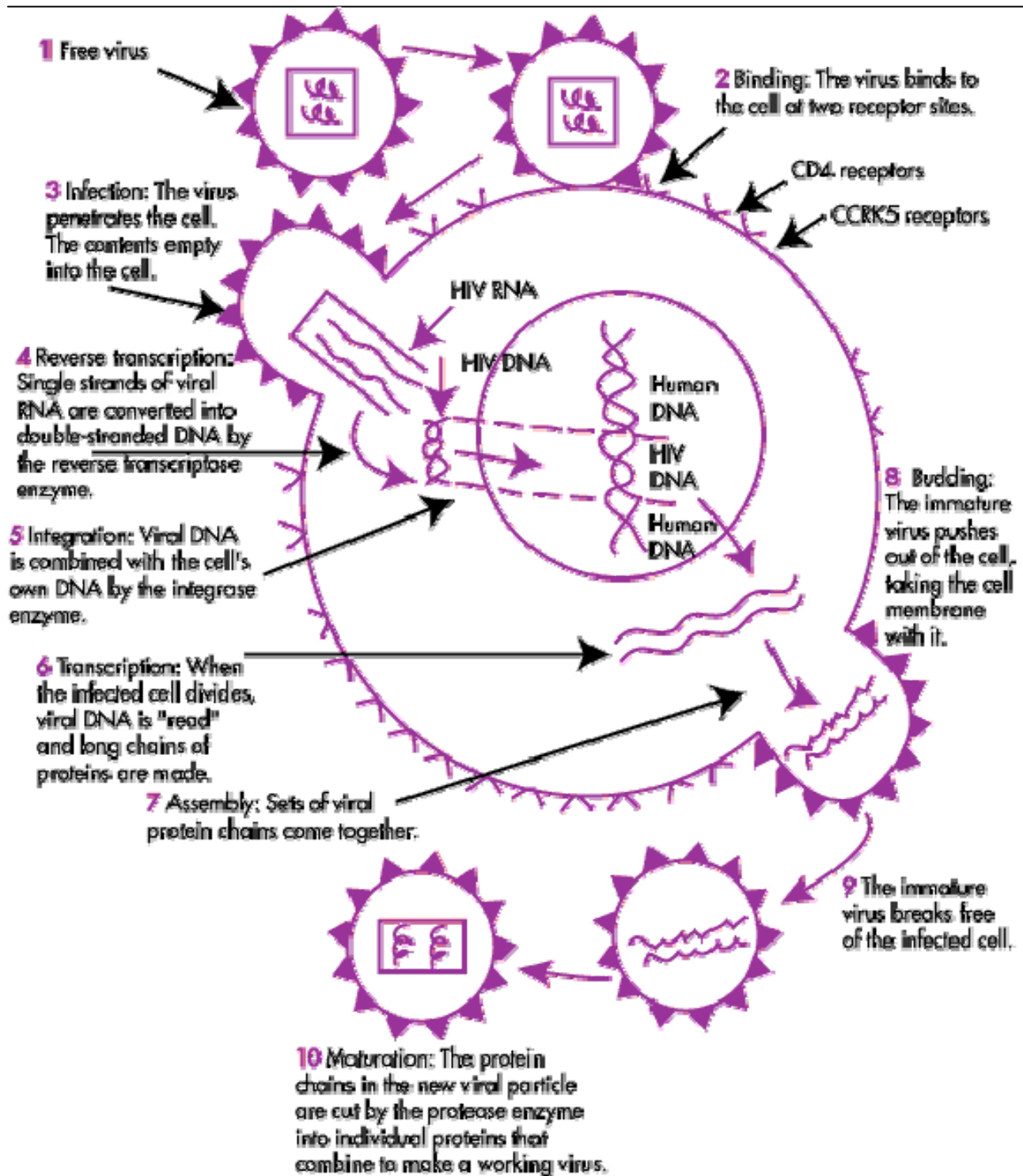
The leukocytes circulate through the body between the organs and nodes by means of the **lymphatic vessels**. Leukocytes can also circulate through the blood vessels. In this way, the immune system works in a coordinated manner to monitor the body for germs or substances that might cause problems.

5. The affects of HIV on the immune system

HIV is a **retrovirus**, a type of virus that stores its genetic information on a single-stranded RNA molecule. After a retrovirus penetrates a cell, it creates a DNA version of its genes, and its DNA becomes part of the infected cell's DNA.

HIV infects one particular type of immune system cell, called **CD4 cells** (or **Tcells**). T-cells coordinate immune regulation and secrete specialized factors that activate other white blood cells to fight off infection. In healthy individuals, the number of CD4 cells normally ranges from 450 to 1,200 cells per microliter of blood (this measurement is known as the *T-cell count*).

When infected with HIV, a T-cell becomes an HIV-replicating cell. In other words, the virus binds with the cell, copies itself into the cell's DNA, and causes the cell to begin producing new HIV viruses. This process eventually causes the cell to die. As the number of T-cells decreases, the infected person's immune system becomes increasingly compromised. When a person's T-cell count (number of CD4 cells) drops to below 200 cells per microliter of blood, the person is considered to have AIDS.



An infected person's body tries to fight off HIV infection by aggressively manufacturing **antibodies**, which are tiny bits of protein designed to bond with HIV particles and neutralize them before they can infect more cells. (Most HIV tests actually detect the presence of these antibodies, not HIV itself.)

It is particularly difficult for the immune system to fight off HIV infection for a number of reasons, including the following:

- HIV attacks the immune system itself, weakening its ability to fight back.
- HIV replicates in large quantities that are more than the compromised immune system can handle.
- HIV has the ability to mutate (change itself) very quickly, making it more difficult for the body to fight the infection.

6. The window period

The window period is the time it takes for your body to produce HIV antibodies after you have been infected by the virus. In most people, this period is between 2 and 12 weeks. In a very small number of people, the process takes up to 6 months.

During the window period, you might be infected with HIV yet still test HIV-negative.



Here's how that can happen. Let's say you have unprotected sex on Saturday night and become infected with HIV. On Monday, you get an HIV test. The test almost certainly will come back negative, because your body has not yet had a chance to make antibodies, which are what the HIV test is looking for.

Even if you go for an HIV test 1 or 2 months later, your results might be negative because your body still has not produced antibodies. It may take 3 months after exposure for the test results to be 97% accurate, and 6 months to be absolutely certain. If you think you have been exposed to HIV, and your test results are negative, be sure to get retested in 3 to 6 months just to be sure.

7. AIDS Symptoms

HIV virus causing AIDS enters the blood and quickly penetrates white cells. Then they program the white cells, after which there is often little or no trace of the AIDS virus at all.

This situation usually lasts for six to twelve weeks. During this time the person is free of HIV or AIDS symptoms and antibody tests for AIDS and HIV are negative.

7.1 First Symptoms of AIDS Illness (HIV Infection)

First thing that starts developing in a flu like illness, which may look like glandular fever with swollen glands in the neck and armpits.

At this stage the blood test will usually become positive as it picks up the tell-tale antibodies. Most people do not realize what is happening, although when they later develop AIDS they look back and remember it clearly. Most people have produced antibodies in about twelve weeks.

Latent infection

The person in this stage has a positive HIV test. The virus often seems to disappear completely from the blood again.

At least nine out of ten who see these HIV and AIDS symptoms will develop further problems.

San Francisco studies show that in developed countries, without use of the latest therapies:

- 50% with HIV develop AIDS in ten years
- 70% with HIV develop AIDS in fourteen years
- Of those with AIDS, 94% are dead in five years

The next HIV AIDS symptoms stage begins when the immune system starts to break down. This is often preceded by subtle mutations in the virus, during which it becomes more aggressive in damaging white cells.

Several glands in the neck and armpits may swell and remain swollen for more than three months without any explanation. This is known as persistent generalized lymphadenopathy (PGL).

Early HIV Virus Progression

As the HIV disease progresses, the person starts showing up other AIDS symptoms. A simple boil or warts may spread all over the body. The mouth may become infected by thrush (thick white coating), or may develop some other problem. Dentists are often the first to be in a position to make the diagnosis. People may develop severe shingles (painful blisters in a band of red skin), or herpes.

They may feel overwhelmingly tired all the time, have high temperatures, drenching night sweats, lose more than 10% of their body weight, and have diarrhoea lasting more than a month. No other cause is found and a blood test will usually be positive.

Some used to call this stage ARC, or AIDS related complex.

Late HIV Illness -The AIDS Symptoms

The final stage is AIDS. Most of the immune system is intact and the body can deal with most infections, but one or two more unusual infections become almost impossible for the body to get rid of without medical help, usually intensive antibiotics.

These infections can be a nightmare for doctors and patients. The desperate struggle is to find the new germ, identify it, and give the right drug in huge doses to kill it.

The germ may be hiding deep in a lung requiring a tube (bronchoscope) to be put down the windpipe into the lung to get a sample. The person is sedated for this. It may be hiding in the fluid covering the brain and spinal cord, requiring a needle to be put into the spine (lumbar puncture). It may be hiding in the brain itself. It may hide in the liver or gallbladder or bowel. It can hide anywhere.

Just to summarize, following are the main HIV symptoms:

- Rapid weight loss
- Dry cough
- Recurring fever or profuse night sweats
- Profound and unexplained fatigue
- Woolen lymph glands in the armpits, groin, or neck
- Diarrhea that lasts for more than a week
- White spots or unusual blemishes on the tongue, in the mouth, or in the throat
- Pneumonia
- Red, brown, pink, or purplish blotches on or under the skin or inside the mouth, nose, or eyelids
- Memory loss, depression, and other neurological disorders

8. Stages of HIV Infection

HIV infects cells in the immune system and the central nervous system. The main type of cell that HIV infects is the T helper lymphocyte. These cells play a crucial role in the immune system, by coordinating the actions of other immune system cells. A large reduction in the number of T helper cells seriously weakens the immune system.

HIV infects the T helper cell because it has the protein CD4 on its surface, which HIV uses to attach itself to the cell before gaining entry. This is why the T helper cell is sometimes referred to as a CD4+ lymphocyte. Once it has found its way into a cell, HIV produces new copies of itself, which can then go on to infect other cells.

Over time, HIV infection leads to a severe reduction in the number of T helper cells available to help fight disease. The process usually takes several years. HIV infection can generally be broken down into four distinct stages: primary infection, clinically asymptomatic stage, symptomatic HIV infection, and progression from HIV to AIDS.

STAGE 1 : Primary HIV Infection**EXAMPLE:**

This stage of infection lasts for a few weeks and is often accompanied by a short flu-like illness. In up to about 20% of people the symptoms are serious enough to consult a doctor, but the diagnosis of HIV infection is frequently missed.

During this stage there is a large amount of HIV in the peripheral blood and the immune system begins to respond to the virus by producing HIV antibodies and cytotoxic lymphocytes. This process is known as seroconversion. If an **HIV antibody test** is done before seroconversion is complete then it may not be positive.

STAGE 2 : Clinically Asymptomatic Stage**EXAMPLE:**

This stage lasts for an average of ten years and, as its name suggests, is free from major symptoms, although there may be swollen glands. The level of HIV in the peripheral blood drops to very low levels but people remain infectious and HIV antibodies are detectable in the blood, so antibody tests will show a positive result.

Research has shown that HIV is not dormant during this stage, but is very active in the lymph nodes. A test is available to measure the small amount of HIV that escapes the lymph nodes. This test which measures HIV RNA (HIV genetic material) is referred to as the **viral load test**, and it has an important role in the **treatment of HIV infection**.

STAGE 3 : Symptomatic HIV Infection**EXAMPLE:**

Over time the immune system becomes severely damaged by HIV. This is thought to happen for three main reasons:

- The lymph nodes and tissues become damaged or 'burnt out' because of the years of activity;
- HIV mutates and becomes more pathogenic, in other words stronger and more varied, leading to more T helper cell destruction;
- The body fails to keep up with replacing the T helper cells that are lost.

As the immune system fails, so symptoms develop. Initially many of the symptoms are mild, but as the immune system deteriorates the symptoms worsen.

STAGE 4 : Progression from HIV to AIDS**EXAMPLE:**

As the immune system becomes more and more damaged the illnesses that occur become more and more severe leading eventually to an AIDS diagnosis.

At present in the UK an AIDS diagnosis is confirmed if a person with HIV develops one or more of a specific number of severe opportunistic infections or cancers. In the US, someone may also be diagnosed with AIDS if they have a very low count of T helper cells in their blood. It is possible for someone to be very ill with HIV but not have an AIDS diagnosis.

9. Opportunistic infections**DEFINITION**

People with advanced HIV infection are vulnerable to infections and malignancies that are called 'opportunistic infections' because they take advantage of the opportunity offered by a weakened immune system.

A partial list of the world's most common HIV-related opportunistic infections and diseases includes:

- Bacterial diseases such as **tuberculosis**, MAC, bacterial pneumonia and septicaemia (blood poisoning)
- Protozoal diseases such as toxoplasmosis, microsporidiosis, cryptosporidiosis, isosporiasis and leishmaniasis
- Fungal diseases such as PCP, candidiasis, cryptococcosis and penicilliosis
- Viral diseases such as those caused by cytomegalovirus, herpes simplex and herpes zoster virus
- HIV-associated malignancies such as Kaposi's sarcoma, lymphoma and squamous cell carcinoma.

Symptomatic HIV infection is mainly caused by the emergence of opportunistic infections and cancers that the immune system would normally prevent. These can occur in almost all the body systems, but common examples are featured in the table below.

As the table indicates, symptomatic HIV infection is often characterised by multi-system disease. Treatment for the specific infection or cancer is often carried out, but the underlying cause is the action of HIV as it erodes the immune system. Unless HIV itself can be slowed down the symptoms of immune suppression will continue to worsen.

System	Examples of Infection/Cancer
Respiratory system	Pneumocystis jirovecii Pneumonia (PCP) Tuberculosis (TB) Kaposi's Sarcoma (KS)
Gastro-intestinal system	Cryptosporidiosis Candida Cytomegalovirus (CMV) Isosporiasis Kaposi's Sarcoma
Central/peripheral Nervous	HIV Cytomegalovirus Toxoplasmosis Cryptococcosis Non Hodgkin's lymphoma Varicella Zoster Herpes simplex
Skin	Herpes simplex Kaposi's sarcoma Varicella Zoster

10. How can you get HIV?

You get HIV from...

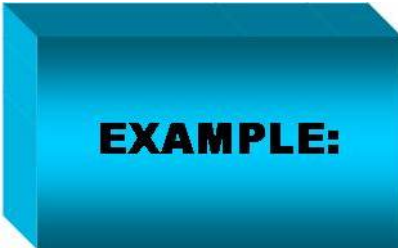
There are three ways in which HIV is passed from one person to another. These are:

- By having unprotected penetrative sex (anal and vaginal)



**Not using a condom with a person who is HIV positive.
Please note, oral sex is considered a high risk practice.**

- By getting HIV infected blood into your blood stream.



EXAMPLE:

This can happen by using unsterilized needles, razor blades, dental equipment etc. It can also happen through a blood transfusion, however in South Africa all blood is tested for HIV before it is used.

- From an HIV positive mother to her baby before and during birth and when breastfeeding.



TAKE NOTE

YOU CAN NOT GET HIV FROM:

- Kissing
- Sharing eating utensils
- Eating food prepared by an HIV positive person
- Eating food served by an HIV positive person
- Sharing bathrooms or toilets
- Sneezing
- Hugging
- Social contact
- Condoms
- Contact sport
- Mosquitoes or other insects/animals
- Swimming pools

[illegible]

STUDY UNIT 2

Behaviour

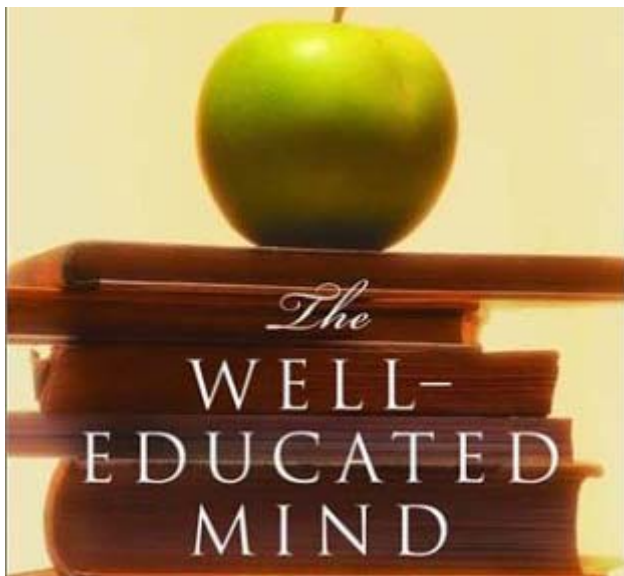
LEARNING OUTCOMES

On completion of this study unit the student will....



- ♦ Explain ways on preventing yourself from getting aids or gets infected by the HIV virus.
- ♦ Identify safe activities.
- ♦ Identify those factors involved when getting tested for HIV.
- ♦ Explain the treatment of HIV.
- ♦ Explain safe sex practices.
- ♦ Identify and explain the risk categories of HIV.

1. How can I prevent myself from contracting HIV?



Becoming educated about HIV and understanding how it is transmitted is the first, and perhaps most important way to prevent the spread of HIV. It is essential for people to make informed decisions about the level of risk they are willing to take, based on what is realistic for them.

Abstaining from sex and needle sharing is the most effective way for people to protect themselves from HIV and other sexually transmitted diseases. However, abstinence is not a realistic option for everyone.

2. Sexual activities

• Safer Sex

When abstinence is not an option, the proper use of barrier protection such as latex or polyurethane condoms (male or female), with a water based lubricant, is the next best thing for vaginal or anal sex.



Some water-based lubricants (including those already on some condoms) contain a spermicide called Nonoxynol-9 (N-9). Many people are allergic to N-9 and the resulting genital irritation can increase the risk of HIV and STD transmission by providing a direct entry point. You can test for a N-9 allergy by rubbing N-9 lubricant on the inside of the elbow the day before you plan to use the product for sex. If there is no irritation, there is likely no allergy.

• Oral Sex

The best way to reduce the risk of HIV transmission while performing oral sex is to maintain good oral hygiene. That, in addition to not flossing or brushing your teeth right before or after will also reduce the risk of transmission.

• Performing Oral Sex on a Woman

When performing oral sex on a woman, a dental dam or common kitchen plastic wrap can be used as a barrier to protect from HIV transmission. It covers the area you are performing oral sex on (vagina or anus). If you do not have a dental dam, you can also use a new, unused, non-lubricated or flavored condom by stretching it out and cutting it down the side, then stretching it out in the same way you would a dental dam or plastic wrap.

- **Performing Oral Sex on a Man**

In addition to good oral hygiene, proper use of a non-lubricated or flavored condom on a man can significantly decrease risk of HIV transmission. If a condom is not available or an option, not accepting semen into the mouth or spitting rather than swallowing will reduce the risk. You can also use the "harmonica method" by focusing on the shaft of the penis while avoiding the head.

3. How Do I Use Male and Female Condoms?

Most male condoms are made of latex. Since some people are allergic to latex (your doctor can test for it if you've ever experienced irritation from latex) there are also polyurethane condoms available. When used properly, both latex and polyurethane condoms are effective ways of significantly reducing the risk of HIV transmission.



*Lambskin, or "natural" condoms **will not** protect against HIV or other sexually transmitted infections (STI's).*

When using either latex or polyurethane condoms for vaginal or anal sex, water based lubricants on the outside of the condom will help to reduce friction that could cause the condom to tear. If desired, a small amount can be placed inside the tip of the condom as well.

3.1 Steps to put a male condom on

- **Steps 1**



Put the condom on when the penis is erect, before there is any contact between the penis and your partner's body. Fluid released from the penis during the early stages of an erection can contain sperm and organisms that can cause STI's.

- Steps 2



Tear along one side of the foil, being sure not to rip the condom inside.



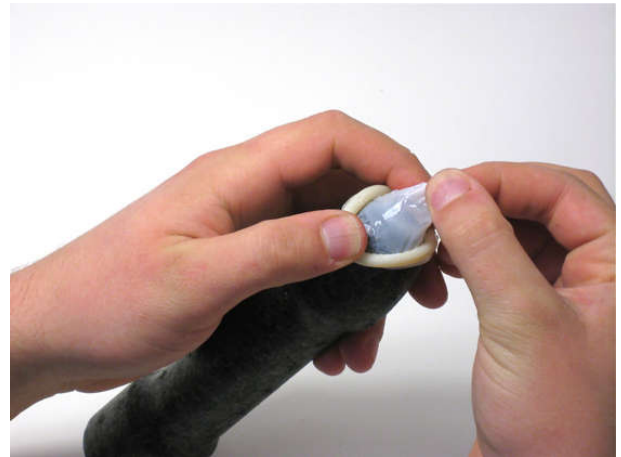
Carefully remove the condom.

- **Steps 3**



Air trapped inside a condom could cause it to break. To avoid this, squeeze the closed end of the condom between your forefinger and thumb and place the condom over the erect penis.

Be sure that the roll is on the outside.



- **Steps 4**

While still squeezing the closed end, use your other hand to unroll the condom gently down the full length of the penis.

Make sure the condom stays in place during sex; if it rolls up, roll it back into place immediately.



**TAKE NOTE**

If the condom comes off, withdraw the penis and put on a new condom before intercourse continues.

3.2. Steps to remove a male condom

Soon after ejaculation, withdraw the penis while it is still erect by holding the condom firmly in place.

Remove the condom only when the penis is fully withdrawn.



Dispose of the used condom hygienically. Wrap the condom in a tissue and place it in a bin (do not flush it down the toilet).



NEVER USE A CONDOM MORE THAN ONCE.

3.3. Condom do's and don'ts

- Open the packet carefully to avoid damaging the condom (jewellery, long fingernails or careless teeth could tear it). The condom comes out rolled up forming a ring which will fit over the cock.
- Put the condom on after the cock is hard and before any kind of fucking begins.
- Check the condom is the right way up; you can test with a finger that it rolls out and down.
- Keep the cock completely free of grease and lubricant for best 'holding power'.
- Squeeze the closed end between the thumb and forefinger to expel the air. Air bubbles can make condoms break.
- Hold the condom over the tip of the cock and with the other hand carefully unroll it down to the base of the cock. It may help to stretch the condom width-ways in order to ease it down the cock and to ensure that it remains in place once fucking begins.
- But be careful not to pierce the condom with your fingernails when doing this.
- If you have a foreskin pull it back before covering the head of the cock with the condom. Don't try to cover the balls with a condom.
- Smooth the condom to eliminate any air bubbles.
- Some men withdraw to check the condom, occasionally. If you need more lubrication (and you should use plenty), use more water-based lubricant.
- Never use two condoms at once. This more is likely to lead to breakage because of rubber rubbing against rubber.
- If the cock gets soft during fucking the condom may slip. Fingers held round the base of the condom will help it stay put .
- If the condom does break or slip off during fucking withdraw as soon as you find out! Obviously you will need to use a new one if this happens.
- A condom is more likely to break if the fuck lasts a long time, (longer than 45 minutes). Consider changing the condom during a long sex session
- After coming (ejaculation) withdraw the cock before it becomes soft (otherwise semen could leak out of the condom). To prevent the condom slipping off your cock at this stage, hold it firmly round the base as you withdraw.
- Throw used condoms away. Dispose of them thoughtfully, e.g. wrapped up in tissue paper and then thrown in the rubbish. They shouldn't be flushed down the loo, since this may cause blockages in sewage disposal, according to water companies.
- Never use a condom more than once.
- Never go fucking from one person to another without changing condoms.

4. Needles

Since HIV and hepatitis C are **blood borne viruses** (viruses that exist in and are transported via the bloodstream), any direct blood-to blood contact is a risk for HIV and hepatitis C transmission. Sharing needles or works (cotton, cookers, water, etc.) presents a significant risk for transmitting these blood borne viruses. Whenever possible, it is best for each person to use their own needles and works.

4.1 Disposal of needles

Proper disposal of used needles is important as well. Used needles *should not* be disposed of in the garbage since this creates a risk for anyone handling the trash who may get stuck by an infected needle. Sharps containers are heavy-duty containers used for disposal of needles, and can be acquired through some pharmacies. If a sharps container is not available, an empty plastic liquid detergent or bleach bottle can be used as well. These should then be turned into a designated needle disposal site. For information about acquiring sharps containers, needle disposal sites, or any other questions related to needle handling and safety, please your local **Pharmacy**.

4.2 Needle Safety

The only way to completely avoid potential transmission of blood borne illnesses when using IV drugs is by **not sharing** needles. If people do share needles, cleaning the needles and works properly with bleach and water before and after each person uses them will help reduce the risk.

4.3 Cleaning used Needle

The most effective way to sterilize used syringes is the **3x3x3 method**:

- Fill your syringe with water, shake it, and push out the water. Do it three times.
- Fill your syringe with bleach, shake it, and push out the bleach. Do it three times.
- Fill your syringe again with clean water, shake it, and push out the water. Do it three times.
- If bleach isn't available, you can use soap and clean water, or even just water to clean your works - ANY steps you take to clean syringes before use will reduce your risk of HIV and Hepatitis C transmission.

5. Safe activities:

While certain sexual activities have little or no risk of HIV transmission (i.e. mutual masturbation, protected oral sex, oral to anal contact (rimming), fingering) possibilities sometimes exist for other sexually transmitted infections (STI's) to be transmitted. While HIV is transmitted only by blood, semen, vaginal fluid, and breast milk, other STI's can be transmitted by simple genital skin-to-skin contact or oral sex. For questions or concerns regarding the risks of particular activities, call the AIDS Action Hotline.

6. Treating HIV

There are three key things that can be done to help prevent all forms of HIV transmission. First among these is promoting widespread awareness of HIV and how it can be spread. Media campaigns and education in schools are among the best ways to do this.

Another essential part of a prevention program is HIV counseling and testing. People living with HIV are less likely to transmit the virus to others if they know they are infected and if they have received counseling about safer behaviour. In particular, a pregnant woman who has HIV will not be able to benefit from interventions to protect her child unless her infection is diagnosed. Those who discover they are uninfected can also benefit, by receiving counseling on how to remain that way.

The third key factor is providing antiretroviral treatment. This treatment enables people living with HIV to enjoy longer, healthier lives, and as such it acts as an incentive for HIV testing. It also brings HIV-positive people into contact with health care workers who can deliver prevention messages and interventions. Studies suggest that HIV-positive people may be less likely to engage in risky behaviour if they are enrolled in treatment programs. Nevertheless, it is also possible that widespread availability of treatment may make some members of the wider population less fearful of HIV infection, and hence less willing to take precautions.

We can not cure HIV, but antiretroviral therapy has extended life for HIV positive clients. The goal of antiviral therapy is to lower the viral count in the bloodstream. There are three major classes of drugs.

Despite this progress, these drugs typically lose their effectiveness after a period of treatment because the virus mutates and becomes resistant, much like bacterial resistance to antibiotics. Cross-resistance, or the virus' ability to mutate to be resistant to multiple drugs of the same class, is also a potential problem.

The only real weapons we have against viruses are natural ones, that is antibodies which can destroy viruses and bacteria. The problem with HIV is that the body cannot produce antibodies against the HIV virus. That is how a HIV test if performed, that is not looking for virus but testing for antibodies.

No antibodies have yet been found in a human being that are effective in the long term against HIV. That is why a vaccine will be so difficult to find.

6.1 HIV Treatment Drugs

Our only other hope lies in a drug that could destroy viruses in the body. We have none that is effective. We will undoubtedly find such a drug one day but it is a long, long way off. How do you kill something (HIV virus) that does not breathe, does not need food, does not live and never dies?

Attempts have even been made to flood the bloodstream with small pieces of cell wall (CD4) so the viruses are unable to touch living CD4 white cells. Another method being tried is to inject antibodies ('neutralising') from HIV-positive people to give extra protection to people with AIDS.

There would be one terrible problem with all such potential drugs. If they can be found, they will have to be taken for life. If some cells in the body are already infected, then a drug preventing entry of new viruses into unaffected cells will need to be taken until every reprogrammed cell and its descendants are dead, which could take fifteen years or longer. If we stopped the drug after ten years and a single reprogrammed white cell were to be activated to make more virus particles, the disease could start progressing all over again. This applies also to drugs preventing reprogramming, virus manufacture, or budding from the cell.

7. HIV Testing

When thinking of getting tested for HIV, there are a few important things to consider:

- **Window Period:** The HIV test is looking for antibodies, which are the body's response to having HIV in it. For most people, it takes 6 weeks for the body to produce enough antibodies for a standard HIV antibody test to be accurate. Some people call this the "window period". It is recommended to wait for 6 weeks between an individual's possible HIV exposure and the time they get tested, providing there are no risks within that time period.
- **Anonymous vs. Confidential Tests:** An anonymous test does not require an individual to provide their name at the time of testing, while a confidential test does require a name. In either case, written consent from the patient is the only way the results will be released, and otherwise will be kept private. Doctor's offices use confidential testing while some private clinics and testing sites will still do anonymous HIV testing, although it is becoming increasingly difficult to find.
- **Standard vs. Rapid Testing:** A standard HIV test refers to a blood draw, typically done at a doctor's office. Results are usually received within 5 to 10 business days. Rapid tests (sometimes referred to as OraQuick or OraQuick Advance) involve either a finger prick or a mouth swab. Results are given in about 30 minutes. See below for descriptions of different types of HIV Antibody tests.
- **Cost:** Depending on where you get tested, either insurance will cover the cost, pricing may be based on a sliding-scale which will depend on your income, or you may be able to get a free test.

7.1 Different types of HIV antibody tests

There are a couple of different types of tests being used to help people determine their HIV status. When people say "HIV test", they are usually referring to a standard antibody test, however there are several other screening tests used to detect HIV antibodies.

Standard Test / ELISA: This is the most common HIV antibody test used by most doctors and laboratories and involves a full blood draw which is sent to a laboratory for testing. The extreme sensitivity of the test makes a negative result conclusive (provided the appropriate window period has passed). Sometimes, the test is so sensitive that it will react to other antibodies besides HIV (usually related to auto-immune disorders) - in this case, the lab would perform a Western Blot test. The Western Blot is a HIV-specific confirmation test that will either confirm or rule-out HIV infection. Results are usually given in 5-10 business days.

Ora-Sure: The Ora-Sure test is perfect for anyone who doesn't like blood. A swab is rubbed between the cheek and lower gum to collect an oral mucosal specimen, which is then sent out for testing. It's recommended that you not eat, drink, smoke, etc. 20 minutes before the test is administered. At the lab, an ELISA and if necessary, a Western Blot is performed on the sample. Results are typically given in 5-10 business days.

Ora-Quick / Rapid Test: The Ora-Quick test is a rapid HIV antibody test, providing results in about 30 minutes. The administrator pricks the patient's finger and takes a few drops of blood which will then be placed into a solution to develop. The extreme sensitivity of the Ora-Quick test means that a negative result (after the window period) is conclusive. If the test does react, the patient will receive a "preliminary positive" result, meaning the test reacted to something in the blood, however not necessarily HIV antibodies. To confirm a reactive Ora-Quick test, blood is drawn and sent to a lab for the Western Blot test.

8. Sexual practices: what's safe and what's not

Some sexual practices are indeed safe, and carry no risk of infecting the participants with the HI-virus. However, other practices are associated with a high risk of HIV infection. How safe are your sexual practices?

The type of bodily fluid involved as well as the number of sexual partners you have determines the extent of risk involved in sexual practices.

Since the highest concentration of the HI-virus is found in blood, semen, pre-cum and vaginal secretions, sexual activities involving these bodily fluids are more risky than contact with saliva, tears, urine and sweat, that have low concentrations of the HI-virus.

Obviously the risk will increase with the numbers of partners you have who are, or have been, involved in unprotected sexual practices.

9. Risk categories

9.1 No Risk

The following sexual behaviour involves no risk and can be regarded as safe:

- Abstinence
- Erotic massage
- Hugging
- Body rubbing
- Kissing
- Petting
- Showering or bathing together
- Masturbation
- Mutual masturbation - providing there is no contact between broken skin and semen or vaginal fluids
- Thigh sex
- Sexual fantasies
- Phone sex
- Using your own personal sex toys



If this describes the sum total of your sexual practices, you do not need to worry about HIV/Aids.

9.2 Low Risk

The following sexual behaviour is regarded as safe, but is still associated with a low risk of HIV/Aids:

- Oral sex on a man (fellatio) who is wearing a condom.
- Oral sex on a woman (cunnilingus) with a latex barrier.
- Anilingus (oral-anal sex) with a latex barrier.
- Contact with urine ("golden showers" or "water sports" on unbroken skin).

If you participated in one of these acts, you are still practicing "safer" sex, but with no absolute guarantee of zero risk. Opt for an HIV-test to put your mind at rest, and try to avoid "Some Risk", "High Risk" and "Very High Risk" behaviors.

9.3 Some risk

These sexual acts are associated with some risk of HIV/Aids. They can be regarded as "fairly safe", but not without some risk:

- Oral sex (on a man or woman) without a condom or latex barrier.
- Vaginal penetration with a condom.

Anal penetrative sex with a condom. It is safer to withdraw before ejaculation. Is this an accurate description of your sex life? Then you are at some risk of contracting HIV/Aids. Opt for an HIV-test to put your mind at rest, and try to avoid "High Risk" and "Very High Risk" behaviour.

9.4 High risk

Unfortunately, many sexual practices are associated with a high risk of HIV/Aids. These include:

- Vaginal penetration without a condom
- Swallowing semen
- Sharing uncovered sex toys
- Vaginal or oral penetrative sex with a condom if using a petroleum-based lubricant
- Unprotected oral contact if blood is present
- Unprotected manual-anal intercourse (fisting) without a latex glove
- Unprotected manual-vaginal intercourse (fisting) without a latex glove
- Contact with menstrual blood

Many people still don't realise that these practices are risky. If you have been involved in any of these acts with anybody else than a very faithful partner, an HIV/Aids test is highly recommended. You may be one of the lucky ones and still be HIV-negative. You may not be. But even then, try to avoid "High Risk" behaviour in future: there is more than one HI-virus and there is no need to get them all. With good nutrition and a healthy lifestyle, many people live with the virus for 15 years or longer.

9.5 Very High Risk

There is sexual behaviour associated with a very high risk of HIV/Aids. These include:

- Anal penetrative sex without a condom.
- Unprotected anal contact if blood is present.

If this is a true description of your sexual behaviour, you are at an extremely high risk to contract HIV/Aids. A test to determine your HIV status is essential, as well as real changes to your sexual behaviour. Since there is more than one HI-virus, try to avoid "High Risk" and "Very High Risk" behaviour. With good nutrition and a healthy lifestyle, many people live with the virus for 15 years or longer.

10. Fears and misunderstandings of HIV

A person in the workplace must take note of the following.

A person in the workplace can not get AIDS from.....

- Kissing
- Sharing eating utensils
- Eating food prepared by an HIV positive person
- Eating food served by an HIV positive person
- Sharing bathrooms or toilets
- Sneezing
- Hugging
- Social contact
- Condoms
- Contact sport
- Mosquitoes or other insects/animals
- Swimming pools

[illegible]

STUDY UNIT 3

Data and trends of HIV

LEARNING OUTCOMES

On completion of this study unit the student will....



- ♦ Explain the trends in spreading HIV in the RSA regions.
- ♦ Explain where reliable HIV statistics can be obtained.
- ♦ Understand the trends and data of HIV and the impact that it have on the country and economic environment.
- ♦ Understand the impact of HIV on the business environment.

1. Introduction

The statistics discussed here come from two prevalence studies that estimate how many South Africans have HIV, and two reports on AIDS deaths.

Viewed together these sources give an idea of the scale of South Africa's HIV epidemic.

The first section is based on the report of the Department of Health "**National HIV and Syphilis Sero-prevalence Survey in South Africa 2006**", published in 2007. This is the 17th in a series of annual studies which look at data from antenatal clinics and use it to estimate HIV prevalence amongst pregnant women.

The second section is based on the report of the "**South African National HIV Prevalence, HIV Incidence, Behaviour and Communication Survey, 2005**". In this survey, a sample of people were chosen to represent the general population, 55% of whom agreed to give a blood sample to be anonymously tested for HIV. The report contains estimates of HIV prevalence in various groups of people, derived from this general population sample.

Seen together, the two prevalence studies provide a clearer picture of the South African epidemic than either of them viewed alone.

The third section looks at AIDS-related deaths using data from death certificates. Three reports published by Statistics South Africa contain the raw data, while the article "**Identifying deaths from AIDS in South Africa**" analyses a large sample of death certificates and attempts to estimate how many deaths caused by HIV have been misclassified.

2. The South African Department of Health Study, 2006

Based on its sample of 33,033 women attending 1,415 antenatal clinics across all nine provinces, the South African Department of Health Study estimates that 29.1% of pregnant women were living with HIV in 2006. The provinces that recorded the highest HIV rates were KwaZulu-Natal, Mpumalanga and Free State.

Until 1998 South Africa had one of the fastest expanding epidemics in the world, but HIV prevalence now appears to have stabilized, and may even be declining slightly. Among teenage girls, the rate fell from 15.9% in the 2005 survey to 13.7% in 2006, possibly indicating a drop in the rate of new infections. Nevertheless it should be noted that the 2006 study involved twice as many women as previous surveys, and samples were collected from more than three times as many clinics; this rather major change in the study population may have influenced the results, as the newly included clinics may have been located in areas with lower HIV prevalence.

Province	2001 Prevalence %	2002 Prevalence %	2003 Prevalence %	2004 Prevalence %	2005 Prevalence %	2006 Prevalence %
KwaZulu Natal	33.5	36.5	37.5	40.7	39.1	39.1
Mpumalanga	29.2	28.6	32.6	30.8	34.8	32.1
Free State	30.1	28.8	30.1	29.5	30.3	31.1
Gauteng	29.8	31.6	29.6	33.1	32.4	30.8
North West	25.2	26.2	29.9	26.7	31.8	29.0
Eastern Cape	21.7	23.6	27.1	28.0	29.5	29.0
Limpopo	14.5	15.6	17.5	19.3	21.5	20.7
Northern Cape	15.9	15.1	16.7	17.6	18.5	15.6
Western Cape	8.6	12.4	13.1	15.4	15.7	15.2
National	24.8	26.5	27.9	29.5	30.2	29.1

Estimated HIV prevalence among antenatal clinic attendees, by age

Age group (years)	2001 Prevalence %	2002 Prevalence %	2003 Prevalence %	2004 Prevalence %	2005 Prevalence %	2006 Prevalence %
<20	15.4	14.8	15.8	16.1	15.9	13.7
20-24	28.4	29.1	30.3	30.8	30.6	28.0
25-29	31.4	34.5	35.4	38.5	39.5	38.7
30-34	25.6	29.5	30.9	34.4	36.4	37.0
35-39	19.3	19.8	23.4	24.5	28.0	29.6
40+	9.8	17.2	15.8	17.5	19.8	21.3

Because infection rates vary between different groups of people, the findings from antenatal clinics cannot be applied directly to men, newborn babies and children. This is why South Africa has sought also to survey the general population.

3. The South African National HIV Survey, 2005

The National HIV Survey is a "household" survey. This involves sampling a proportional cross-section of society, including a large number of people from each geographical, racial and other social group. The researchers take great pains to try to make the sample as representative as possible, and the findings are later adjusted to correct for likely over- or under representation of individual groups (according to census data).

The survey's fieldworkers visited 12,581 households across South Africa, of which 10,584 (84%) took part in the survey. Of the 24,236 people within these households who were eligible to take part, 23,275 (96%) agreed to be interviewed and 15,851 (65%) agreed to take an HIV test. This means that only 55% of eligible people were tested.

The main reasons given for refusing HIV testing were fear of having a blood sample taken (58%); religious objections to having a blood sample taken (16%) and not wanting to learn HIV status (7%). A further 13% of people who refused were, for various reasons, afraid or mistrustful of the survey. The report of the survey claims that people at high risk for HIV infection were more likely to take part, and the results were adjusted to compensate for this perceived bias.

Estimated HIV prevalence among South Africans, by age

Age (years)	Male Prevalence %	Female Prevalence %
2-4	4.9	5.3
5-9	4.2	4.8
10-14	1.6	1.8
15-19	3.2	9.4
20-24	6.0	23.9
25-29	12.1	33.3
30-34	23.3	26.0
35-39	23.3	19.3
40-44	17.5	12.4
45-49	10.3	8.7
50-54	14.2	7.5
55-59	6.4	3.0
60+	4.0	3.7
Total	8.2	13.3

Among females, HIV prevalence is highest in those between 25 and 29 years old; among males, the peak is in the group aged 30-39 years. According to these results, males aged 15-49 years old are 58% as likely to be infected as are females in the same age group (11.7% in men versus 20.2% in women).

4. Studies of AIDS deaths

4.1 All reported deaths

In June 2007, Statistics South Africa published the report "Mortality and causes of death in South Africa, 2005". This large document contains tables of how many people died from each cause according to death notification forms.

This report, alongside a previous edition published in May 2006, reveals that the annual number of registered deaths rose by a massive 87% between 1997 and 2005. Among those aged 25-49 years, the rise was 169% in the same nine-year period. Part of the overall increase is due to population growth. However, this does not explain the disproportionate rise in deaths among people aged 25 to 49 years. In 1997, this age group accounted for 30% of all deaths, but in 2005 it accounted for 42%.

Reported deaths from all causes, 1997 to 2005

Year of death	Age (years)					Total
	0-9	10-24	25-49	50+	Unspecified	
1997	35.441	22.636	92.796	160.058	5.574	316.505
1998	41.172	25.799	114.215	178.763	5.104	365.053
1999	41.834	27.686	129.881	178.877	2.704	380.982
2000	42.802	29.463	150.149	189.118	2.204	413.736
2001	44.876	31.408	172.963	201.738	1.911	452.896
2002	50.741	34.381	200.844	211.504	2.024	499.494
2003	56.593	37.363	228.819	227.280	2.770	552.825
2004	62.212	38.054	242.066	222.231	2.925	567.488
2005	67.559	38.221	250.043	232.168	3.222	591.213
Increase 1997-2005	91%	69%	169%	45%	-42%	87%

The influence of population growth can be removed by looking at death rates per 100,000 people, which are provided by Statistics South Africa in another report called **"Adult mortality (age 15-64) based on death notification data in South Africa: 1997-2004"**. These data show that between 1997 and 2004, the death rate among men aged 30-39 more than doubled, while that among women aged 25-34 more than quadrupled. The changes are even more pronounced when deaths from natural causes only are examined. Over the same period there was relatively little change in the death rates among people aged over 55 and those aged 15-20. In their report, Statistics South Africa call such developments "astounding", "alarming" and "disturbing".

4.2 Misclassification

In 2004, HIV was recorded as a cause of death in only 13,590 cases. However, according to researchers from the Medical Research Council of South Africa (MRC), this figure is a massive underestimate, because the majority of deaths due to HIV are misclassified.

People whose deaths are caused by HIV are not killed by the virus alone, but HIV should be recorded as an underlying cause if it "initiated the chain of morbid events leading directly to death". In other words, if someone contracts tuberculosis and dies from it because their immune system has been weakened by HIV then HIV should be included among the underlying causes. The MRC researchers claim that in many cases, this does not happen; instead, the doctor records only the immediate cause of death such as tuberculosis or respiratory infection. This could be because the doctor does not know the deceased person's HIV status. Alternatively, they may seek to conceal HIV infection to spare stigmatization of relatives, or to avoid invalidating life insurance claims. As The Lancet notes, authorities are largely to blame:

"Social stigma associated with HIV/AIDS, tacitly perpetuated by the Government's reluctance to bring the crisis into the open and face it head on, prevents many from speaking out about the causes of illness and deaths of loved ones and leads doctors to record uncontroversial diagnoses on death certificates.... The South African Government needs to stop being defensive and show backbone and courage to acknowledge and seriously tackle the HIV/AIDS crisis of its people."

TAKE NOTE

From the above statistics one can easily analyse the impact that the growing figures have on the security industry. Looking at the age factor where aids is causing the most deaths it is highly visible in the early twenty's. This means that it becomes more and more difficult to recruit and train new recruits. One also needs to keep in mind that with the growing crime rate in the country security forces are now the fastest growing industry in the country and creates the most jobs for the average person.

What makes it even more crucial is that security companies can not afford the cost involved and the time frame it takes to constantly train new recruits. For this reason you have take note of the impact that the statistics will have on the average company.

5. The impact of HIV / AIDS on all businesses

HIV/AIDS impacts on all businesses both directly and indirectly, resulting in reasons for increased business costs include:

Recruitment and training: The more skilled and unskilled employees you lose due to HIV/AIDS, the more time and money you need to spend on recruiting and training new staff.

Higher pension fund and insurance premiums: The more employees you lose due to HIV/AIDS, the more likely your premiums will increase.

Administration: As more of your employees become affected by or infected with HIV, the greater your administrative work. In some companies, a full-time person/team may be needed to handle the extra administrative work.

Medical care: As more of your employees become infected by HIV, your medical aid premiums will increase. If your company offers ill employees other kinds of healthcare support, your expenses in this regard will also climb.

5.1 Reduced productivity

Reasons for reduced productivity include:

- Employees who are ill due to HIV/AIDS will need more sick leave and/or work less efficiently.
- Training new employees to fill your skills gap, caused by employees lost or absent due to HIV/AIDS, is unavoidably time consuming and slows down productivity.
- Employees worrying about family or friends who are affected by or infected with HIV/AIDS will not be 100% focused on their work.
- Employees having lost family and friends due to HIV/AIDS will require more compassionate/funeral leave.
- High staff turnover and the frequent loss of co-workers decreases staff moral and motivation.

6. The impact of HIV on the private security industry

The study have been initiated by SASSETA. The findings have been recorded in a journal which is available at the offices of SASSETA. The findings below is the main findings as summarized in the executive summary.

6.1 THE HIV PREVALENCE, HIV INCIDENCE AND KABP SURVEY

Private security sector

The following results were obtained:

1. HIV prevalence among the respondents in the private security sector is 15,9%. Other analysis showed that:
 - ♦ Males had a slightly (although not significantly) higher HIV prevalence (17,3%) than females (12,3%);
 - ♦ Africans had a substantially higher prevalence (27,3%) than other race groups (less than 1%);
 - ♦ Respondents aged 25—49 years had a higher HIV prevalence (17.9%) than respondents aged 50 years and older (7,5%) and respondents aged 24 years and younger (7,3%);
 - ♦ Widows had the highest HIV prevalence (29,4%), and divorced or separated people had the lowest HIV prevalence (6,8%);
 - ♦ KwaZulu-Natal (22,8%) had the highest HIV prevalence, followed by Gauteng (17,8%) and the Western Cape (3,4%);
 - ♦ Respondents who were labourers, cleaners, porters and messengers had the highest HIV prevalence (24,5%), followed by service workers, clerks and protective service workers (21,8%), while senior officials, professionals, managers and directors had the lowest HIV prevalence (5,1%);
 - ♦ HIV incidence was higher among Africans (2,5%) than other race groups;
 - ♦ HIV incidence among respondents younger than 25 years of age (3,6%) was higher than among those aged 25—49 years (1,2%);
 - ♦ No new infections found among respondents aged 50 years and older; and
 - ♦ Respondents from KwaZulu-Natal had a higher HIV incidence (3,4%) than those from the Western Cape (1,1%) and Gauteng (2,0%).
2. The respondents in this study were generally very knowledgeable about HIV/ AIDS, except for the following few misconceptions or myths:
 - ♦ Patients with TB also have HIV;
 - ♦ Once one has started taking antiretroviral treatment for HIV/AIDS one has to take it forever;
 - ♦ There is a cure for AIDS; and
 - ♦ Sharing a cigarette (as well as coughing and sneezing) spreads HIV.
3. Generally positive attitudes towards PLWHA were found on most issues, except for the following two, about which many respondents were either negative or ambivalent:
 - ♦ Having protected sex with a partner who is living with HIV/AIDS; and Disclosing the status of a family member, which most respondents indicated they would want to keep a secret or were unsure about disclosing.

4. The overwhelming majority of participants (95.6%) had started taking the AIDS problem more seriously, and this was equally true for both sexes and the different race groups. Two-thirds of respondents (60%) indicated that it was because of the increased number of deaths due to AIDS, while almost a third (30,1%) viewed the reality of the disease as the second main reason.
5. More males than females believed that they were at risk of HIV infection. Conversely, more females than males believed that they were not at risk. Most believed they were not at risk because they were faithful to one partner/ trusted their partner, either always used condoms or were abstaining from sex, did not share used needles or body-piercing instruments, did not have sex with prostitutes, and knew that both they and their partner had tested HIV negative, in that order.
6. Of the four race groups, both males and females of African origin (95.3% and 89.7% respectively) were found to be the most sexually active in the last 12 months, compared with their counterparts from the other race groups, especially white males (90.8%) and coloured females (76.3%).
7. The large majority of respondents (86.7%) reported that they had regular sexual partners, 10.4% had non-regular sexual partners and 0.6% had had sex with commercial sex partners. The breakdown of those who had non-regular partners was as follows:
 - ♦ More African and coloured males reported having had two or more sexual partners than did their male white and Indian/Asian counterparts;
 - ♦ More coloured females reported having had two sexual partners than did their counterparts from the other race groups; and
 - ♦ More importantly, not a single Indian/Asian female reported having had two or more concurrent sexual partners.
8. The large majority of respondents (89%) of both sexes had partners who were within 10 years of their own ages. The breakdown was as follows:
 - ♦ More males (10%) than females (3.3%) reported that they had sexual partners who were 10 years younger than themselves; and
 - ♦ More females (8%) than males (0.5%) reported having had sexual partners who were 10 years older than themselves; and
 - ♦ More Africans (10%) had sexual partners who were 10 years younger than themselves, when compared to other groups, with white respondents coming a close second (6.7%).
9. With regard to condom use:
 - ♦ Nearly a third (32.9%) of all the respondents who agreed to be interviewed had ever used condoms;
 - ♦ Among those sexually active, 41.9% reported having used condoms with regular partners in the past 12 months, 7.6% with non-regular partners, and 0.5% with sexual partners who engaged in commercial sex;
 - ♦ Both males and females below 25 years of age reported relatively high levels of condom use (62.4% and 53.6% respectively), compared to their counterparts of 50 years and above (16.4% of males and 9.1% of females respectively);
 - ♦ Both single males and females reported higher use of condoms (57.1% and 53.0% respectively) than did those of other marital statuses;
 - ♦ Widowed males reported the highest condom use, after single males, while none of the widows reported having used condoms in the last 12 months;

- ♦ African males and females reported significantly higher use of condoms (46.0% and 49.8% respectively) than did white and coloured males (23.8% and 23.3% respectively) and white and coloured females (22.8% and 22.6% respectively);
 - ♦ Both male (63.9%) and female (44.8%) respondents with two partners reported higher condom use than their counterparts with single partners only (37.0% and 31.9% respectively);
 - ♦ Single respondents across all three age groups reported significantly higher use of condoms in their last sexual intercourse, compared to their married, widowed or divorced counterparts; in particular, respondents below 25 years of age (65.4%) reported higher use of condoms than did respondents in the other two age groups (53.2% for those aged 25—49 years and 50.0% for those older than 50 years); and
 - ♦ Respondents younger than 50 years old who had more than two partners reported the highest condom use in their last sexual encounters.
10. The large majority of respondents (88%) knew where to obtain VCT services. However, white respondents and those aged 50 years and older were least aware of VCT centres. With regard to VCT use:
 - ♦ A slight majority of respondents (53%) had ever had an HIV test, of whom 95% had been told of their test results;
 - ♦ The majority of those tested (70.2%) had pre-test counseling before undergoing the test, with more men (73%) than women (64%) having received counseling;
 - ♦ Fewer, but still a majority of the respondents (60%), had post-test counseling after having had an HIV test; and
 - ♦ Female respondents (64%) were more likely than males (46%) to report having being aware of their HIV status.
 11. Half of the respondents (48.2%) who knew about their HIV status had regular partners, and the large majority (78.8%) of those with non-regular partners indicated that they had used condoms consistently in the past 12 months.
 12. One-fifth of the respondents (18.5%) who were found to be HIV-positive in this study had been tested for HIV within the previous two years, while 16.3% had undergone HIV testing more than two years previously.
 13. Using a 10-item international Alcohol Use Disorder Identification Test (AUDIT) to assess alcohol use, low-risk alcohol use was most common among coloured respondents and in the Western Cape.
 14. A very low level of drug use was found in this study, with dagga (cannabis) being the most commonly used drug (and then amongst only 1.3% of respondents).
 15. Two-thirds of the respondents (67%) reported having changed their behaviour in the face of widespread HIV infection, using mainly ABC strategies such as having one partner only or being faithful, always using condoms, abstaining from sex, or reducing their number of sexual partners.
 16. Of the 1 582 males who participated in this part of the study, 40.5% reported having been circumcised. No reliable difference was found between the HIV prevalence rates of men who had been circumcised (17.2%, 95% CI = 12.55—23.09) and those who had not (17.4%, 95% CI = 13.05—22.84).
 17. The most common message/slogan recalled was on condom used, followed by abstinence tied with 'fear', and the need for faithfulness.

18. The majority of respondents were generally comfortable communicating about sex, sexuality and HIV/AIDS-related issues, except for females talking with their colleagues, presumably males, about sexual matters.
19. Respondents who reported having been sexually active in the previous year had the highest HIV prevalence (16.7%), followed by those who reported secondary abstinence (10.9%) and those who claimed to be virgins (1.6%).
20. The HIV prevalence rate was higher among sexually active respondents who reported having had one sexual partner (16.4%) or multiple concurrent sexual partners (17.0%) than it was among those who practiced secondary abstinence (10.9%). However, there were no significant differences in HIV prevalence among respondents with various types of sexual partnerships.
21. HIV prevalence was found to be higher among respondents who reported that they had used condoms during their last sex act than it was among those who reported not having done so. More HIV-positive respondents reported that they had been using condoms with regular sexual partners consistently over the past year, compared to those who had not.
22. Respondents who perceived themselves to be at high risk of HIV had a higher HIV prevalence (19.9%) than those who considered themselves to be at low risk (9.8%). However, the difference was not significant.
23. Males who had partners 10 years younger than themselves had a higher HIV prevalence (20.2%) than males who had partners 10 years older than themselves (10%). However, females who had partners 10 years older than themselves had an HIV prevalence of 16.3%, compared to a prevalence of 9.3% among those that had partners 10 years younger than themselves.

6.2 Employer perceptions of the impact of HIV/AIDS on business

The following main findings were obtained from the two studies:

- ♦ Just over half of the employer respondents (53.8%) regarded HIV/AIDS as a business concern. Among these, the majority were large employers with 200 employees and more.
- ♦ Overall, very little was happening in terms of HIV/AIDS activities among participating companies.
- ♦ Nearly half of the respondent companies (46.8%) reported a small impact on company operations. Nearly a third (30.8%) reported that they anticipated a large impact in the next three years, with 7.7% anticipating a large impact on profits, 23.1% a moderate impact and 30.8% a small impact.
- ♦ Most employer respondents reported that HIV/AIDS had no impact on their employee profile. However, a few indicated otherwise, and the impact varied by occupational category, especially among service workers, security guards and labourers.
- ♦ Many AIDS or AIDS-related deaths were reported to have occurred in 2003 and 2004.
- ♦ Turnover was mainly among service workers and security workers, and a fair number was reported among labourers.
- ♦ Most companies provided a company retirement benefit, whereas only half provided either a medical aid or funeral benefit. Very few reported the provision of HIV/AIDS coverage or an occupational health clinic, the latter in lieu of medical coverage.
- ♦ Most companies did not anticipate that HIV/AIDS would have much of an impact on increasing employee benefit costs. On average, 66.7% reported that there would be no impact on benefit costs.
- ♦ Companies spent very little on HIV/AIDS services in the period prior to the survey. However, there does appear to have been some expenditure on HIV/AIDS education and awareness services and VCT.
- ♦ Most companies felt that HIV/AIDS had no impact on skills demand and supply across all occupational categories. Overall, it appears that there had been relative stability in the companies in regard to the demand and supply of skills, irrespective of occupation; consequently, very few companies had strategies in place to deal with potential labour and skills turnover.
- ♦ Most companies felt that HIV/AIDS had no major impact on investment in training. Among those that indicated otherwise, increases had been among labourers (33%), followed by service workers and learners (20% each), the occupational categories reported by companies as being severely impacted on by HIV/AIDS.
- ♦ While some companies reported that there had been no HIV/AIDS impact on output and service delivery, some indicated that there had been increases in sickness-related absenteeism and funeral attendance (38.5%) and health-related turnover (25%), while others (23.1%) reported decreases in the availability of critical skills or experience and declines in workforce morale. Also, declines in labour productivity appeared to be slight (15.4%) in these companies. Finally, consumer demand appeared to have experienced no impact.

6.3 Employer perceptions of business response

The following four main findings were obtained:

- ◆ Most companies had very little knowledge or awareness of industry-wide or union-based HIV/AIDS policies.
- ◆ Nearly a third of the companies, in particular the larger ones, had a written HIV/AIDS policy, and two-thirds of these had implemented the policy in their workplaces.
- ◆ In spite of the absence of HIV/AIDS policies, some companies, nevertheless, were implementing HIV/AIDS programmes or elements thereof, such as the replacement of staff that became ill (50%), condom provision (41.7%), job security for HIV-positive staff (41.7%), prevention and awareness education (38.5%) and provision of protective equipment (33.3%).
- ◆ Most companies were not aware of the reasons for poor implementation, while some cited the nature of the 'unskilled' workforce and 'long distances' or multiple worksites as obstacles in conducting education and awareness programmes in the sector.

6.4 Employee perceptions of HIV/AIDS impact

The following findings about employees emerged from data collected as part of Study 1:

- ◆ Very few employee respondents displayed knowledge about colleagues living with HIV/AIDS (4.1%) or having died of HIV/AIDS-related illnesses (9.6%).
- ◆ In spite of this lack of knowledge about colleagues living with HIV/AIDS or having died of HIV/AIDS-related illnesses, more than half of all respondents reported that employees had been affected in terms of taking over the tasks of colleagues who were ill (64.9%), a shortage of employees (58.4%) and decreases in effective functioning (58.7%).
- ◆ Most employees displayed very low levels of awareness of HIV/AIDS policies, with only 8.5% aware of an industry-wide policy, which was considerably higher than the 2.5% awareness of a trade union policy. Interestingly, two-thirds of those who were aware of an industry-wide policy indicated that it had been implemented in their companies.
- ◆ The most commonly cited contents of company HIV/AIDS policy (among those few respondents who were aware of it) were non-discrimination, confidentiality, safety in the workplace, counseling, support of employees living with HIV/AIDS, and prevention programmes.
- ◆ Very few of the respondents were able to identify gaps in the HIV/AIDS policies, which included counseling and support of PLWHA, workplace HIV/AIDS prevention programmes, and HIV testing of employees.
- ◆ With regard to reported employee access to HIV/AIDS interventions, the most commonly cited interventions were the provision of equipment to protect staff from blood infections, condom provision, prevention programmes and replacement of staff when ill.
- ◆ Amongst the key programmatic gaps identified were lack of education and training about HIV/AIDS (42.1%), followed by the absence of VCT programmes (28.7%) and programmes to address stigma (14.2%).

The following recommendations are made:

- In order to remain competitive in the long run, the private security sector needs to take a long-term developmental attitude and approach to maintaining and extending the productive lives of its skills base, which is comprised mainly of security guards and other protective service workers.
- The pre-existing awareness of HIV/AIDS as a business concern must be maintained and up-scaled. A major networking approach should prioritize working with sectors that have highly mobile workforces, in order to transmit the HIV/AIDS message effectively across multiple worksites. Also, co-ordination with clients on health and safety measures and VCT provision for subcontracted guards is necessary.
- The low spending on HIV/AIDS services and non-responsiveness among SMMEs could be addressed through discretionary grant funding of low-cost prevention and education programmes.
- Company-based assessments may be encouraged through the use of proxy tools such as anonymous VCT services, which measure and establish individual HIV status.
- The impact of HIV/AIDS should be regarded as a key co-factor in the implementation of Sector Skills Plans (at a macro-level) and Workplace Skills Plans (at a micro-level), as HIV/AIDS-related turnover appears to be concentrated in key feeder occupations that lead to skilled employment. In protecting its massive investment in the development of human capital, SASSETA needs to shift its constituent members away from an attitude of complacency towards con-scientisation and effective intervention.

[illegible]

STUDY UNIT 4

Legislation and policy

LEARNING OUTCOMES

On completion of this study unit the student will....



- ♦ Identify and explain legislation that have an impact on the workplace concerning HIV/AIDS.
- ♦ Explain the importance of developing a HIV workplace policy.
- ♦ Use the guidelines to develop a workplace HIV policy.

1. Introduction

Many employers and employees do not know that the Constitution includes human rights for people living with HIV/AIDS. As a result of ignorance, employers or employees may unwittingly do things that are in fact illegal and can have serious consequences.

To avoid legal problems in the workplace and ensure that employers and employees act in accordance with the law, this tool gives you the relevant information. We're talking about:

- A human rights and legal framework needed when dealing with HIV/AIDS issues in your company.
- Information that you should pass on to all your employees.

2. Human rights and laws regarding the handling of HIV/AIDS issues in the workplace

Generally speaking, many people living with/affected by HIV/AIDS are discriminated against and stigmatized both in the workplace, and in their communities. However, it is a constitutional law that people living with/affected by HIV/AIDS may not be discriminated against. What's more, they have the right to treatment, care and support services.

Currently there are five Acts of Parliament, which impact on the workplace. These Acts are:

- The Basic Conditions of Employment Act
- The Labour Relations Act
- The Compensation for Occupational Injuries and Diseases Act
- The Occupational Health and Safety Act
- The Employment Equity Act

However, the two laws dealing with HIV/AIDS and employment are **The Labour Relations Act and the Employment Equity Act.**

2.1 The Labour Relations Act

This act makes it illegal to dismiss anyone for illness ("incapacity") without doing the following:

- Firstly, finding out whether that employee is indeed permanently unable to work
- Secondly, making every effort to try and place that employee in a job that he/she can cope with.

2.2 The Employment Equity Act

This act makes it illegal for an employer to do the following:

- Force an employee or prospective employee to have an HIV Test without first getting permission from the Labour Court. Only under very specific circumstances, will the Labour Court give such permission.
- An employer cannot refuse to employ an HIV positive person, or dismiss an employee because of his/her HIV status.



- **An employee with HIV is not required to reveal his/her HIV status** to the employer. However, if an HIV positive employee does disclose his/her status, he/she may benefit if management is understanding and provides some form of help, treatment and support.
- If **an employer or co-worker becomes aware** of an employee's HIV status, **they are expected to keep such information confidential**, unless the employee with HIV gives specific consent for the information to be disclosed.
- **No employee may refuse to work with a fellow-employee with HIV**. Such refusal should lead to disciplinary action.

3. Legal compliance

To ensure your company is legally compliant with the relevant Acts and laws, you should do the following:

1. Have a critical and careful look at all your HIV/AIDS policies and procedures (written documents), your workplace culture and the typical actions of management and your employees. Remember, everything you do must be based on human rights principles and comply with the law.
2. Ensure that your working environment protects the legal and human rights of all employees and that you reduce opportunities for possible discrimination and stigmatization by:
 - Starting and supporting activities and programs that make people aware of the rights of people living with HIV/AIDS.
 - Creating an HIV/AIDS workplace policy that does not discriminate against people living with HIV/AIDS.
 - Ensuring that all your employee benefits do not discriminate against people living with HIV/AIDS.
3. Put mechanisms in place for dealing with human rights abuses and instances of discrimination.

4. If you feel that you do not have the resources in-house to check that your current HIV/AIDS policies and procedures are legally compliant, we encourage you to get legal advice from any of the following resources:

- The AIDS Consortium
- The AIDS Law Project
- The AIDS Legal Network
- The South African Human Rights Commission
- The Legal Aid Board

4. Developing an HIV/AIDS workplace policy

More than 92% of all companies operating in South Africa's hospitality sector do not have an HIV/AIDS workplace policy in place. This makes it difficult to standardize and control practices and procedures and ensure employees living with HIV/AIDS are treated in a fair and consistent way.

The purpose of this tool is to help you develop an HIV/AIDS workplace policy by giving you some guidelines as well as sample policies which you can adapt as you see fit.

TAKE NOTE

Irrespective of your company's size, it is very important that you have a clear and comprehensive HIV/AIDS workplace policy in place.

DEFINITION

An HIV/AIDS workplace policy is a document that defines how your company will consistently and fairly treat employees who are living with HIV/AIDS, as well as employees who are indirectly affected by the epidemic.

5. Guidelines for developing an HIV/AIDS workplace policy



Sample policy for all companies



Please adapt the following sample HIV/AIDS workplace policy to suit your company's particular needs.



Policy documents typically contain legal jargon.
However, you can use more user-friendly
language if you wish.

Sample policy for all companies⁸

Please adapt the following sample HIV/AIDS workplace policy to suit your company's particular needs. **Please note:** Policy documents typically contain legal jargon. However, you can use more user-friendly language if you wish.

HIV/AIDS Workplace Policy for _____ *(Insert your company's name)*

1. Policy Purpose:

- a) To ensure that the company uses a consistent approach when dealing with employees who are living with HIV/AIDS.
- b) To ensure that this approach is legally compliant and does not infringe on the constitutional rights of employees.

2. Responsibility:

(Insert the department/ the job title of the person/people responsible for this policy eg. Human Resources, Human Resources Director, Managing Director, Chief Executive Officer, etc)

3. Effective Date:

(Insert the date on which this policy becomes effective. As and when you make any changes to your policy, so you will change this effective date.)

4. Background: *(This section provides a background on HIV/AIDS. Adapt this according to your needs.)*

HIV/AIDS is widespread throughout southern Africa. With the dramatic rise in AIDS deaths, the socio-economic impact on employers, families and communities will become an increasing burden. This will impose a threat to the ability of our company to deliver services to our clients. We recognise this burden and are committed to dealing with this socio-economic problem.

5. Principles:

The company affirms the principles of equality and equity and further states that:

- This policy has been developed and implemented in consultation with employees and their representatives.
- Employees living with HIV/AIDS have the same rights and responsibilities as all other employees. Therefore the company will do everything in its power to protect employees living with HIV/AIDS against discrimination and ensure confidentiality regarding their HIV status.
- Testing for HIV infection will not be imposed on employees (HIV is not classified as a notifiable disease, and transmission through the handling of food is impossible). Where HIV testing is done at the request of the employee, this will be with his/her informed consent, accompanied by counselling and the results will only be available to the employee.

6. Coordination and Management Strategies

- A dedicated HIV/AIDS person will be nominated and situated at _____ *(eg head office)* and supported by an HIV/AIDS task team with representation from *(List applicable department and divisions)*

- Regular _____ *(monthly, biannual, etc)* progress reports on the strategy will be made to the

(e.g Executive Committee/Board of Directors/Senior Management, etc.)

- The leadership of the company will utilise all appropriate opportunities to demonstrate support for the HIV/AIDS workplace policy and strategy.

7. Human Resources Issues

- The company will regularly review employee benefits in the context of the impact of the epidemic. In addition, a skills succession plan will be developed.

• Recruitment

In accordance with Employment Equity legislation, an employee will not be denied employment if he/she is HIV positive, provided that he/she is deemed fit to perform the job for which he/she has applied. Furthermore prospective employees will not be required to undergo HIV testing.

• Medical Benefits

Provide details of your company's Medical Aid and Medical Aid benefits offered to employees i.e. who has access to these benefits, what do these benefits offer in terms of HIV/AIDS, etc

Details of the company's Medical Aid and particular scheme are as follows:

Name of medical aid	Medical aid benefits including HIV/AIDS treatment	Beneficiaries of medical aid benefits

• Public Health Care

If your company does not offer all employees Medical Aid benefits, we recommend that you include information on the nearest public health service/s available to your employees.

The nearest hospital and/or clinic to the workplace that offers HIV/AIDS medical assistance is:

Name of hospital or clinic	Address	Contact telephone number	Hours of operation

• Pension/Provident Fund

Provide details of Pension/Provident Fund benefits offered to employees i.e. who has access to these benefits, what do these benefits offer in terms of HIV/AIDS, etc.

Details of the company's Pension/Provident Fund are as follows:

Name of pension/provident fund	Fund benefits including grants for employees living with HIV/AIDS	Beneficiaries of fund

• Unemployment Insurance Fund

If your company does not offer all employees Pension/Provident Fund benefits, we recommend that you provide information on the nearest Unemployment Insurance Fund (UIF) office.

The nearest UIF office to the workplace is:

Name of UIF office	Address	Contact telephone number	Hours of operation

• Sick Leave

Employees, who require sick leave as a result of illness related to chronic/life threatening diseases, including HIV/AIDS, will be entitled to the same sick leave allocation as all other employees. Information on sick leave is available at/on/in (where?) _____. (Insert where employees can access this information e.g. HR, the intranet, each employee's employment contract.) According to the Basic Conditions of Employment Act, employees are entitled to paid sick leave equivalent to the amount of days worked in a 6 week working period, during a three year cycle.

• Family Responsibility Leave

Employees who require time off work to attend to the needs of immediate family members, who are seriously ill or have died, will be entitled to ____ days family responsibility leave per calendar year. Information of this leave is available at/on/in (where?) _____. (Insert where employees can access this information e.g. HR, the intranet, each employee's employment contract.) According to the Basic Conditions of Employment Act employees are entitled to a minimum of 3 days per year for family responsibility leave.

• Loans

If applicable provide information on your company's loan policy for employees with chronic/life threatening diseases.

8. Legal Compliance

The company undertakes to consider employees living with HIV/AIDS in terms of their contract of employment and fair and reasonable labour practices. The company commits to compliance with the following laws:

- The Employment Equity Act
- The Labour Relations Act
- The Occupational Health and Safety Act
- The Basic Conditions of Employment Act (or employment law applicable to your business)
- The Compensation for Occupational Injuries and Diseases Act

Unfair discrimination is against company policy and procedures and therefore discrimination against an employee on the grounds of their HIV status should be reported to _____ (Managing Director, CEO, Human Resources Manager, HIV/AIDS task team, etc), or go through the company's grievance procedure.

9. Existing Employees

HIV/AIDS is not a notifiable disease and as such employees who contract the disease are not obliged to inform the company. However, there is a duty to notify the company of incapacity or disability once an employee's health deteriorates to such an extent that the employee is unable to perform his/her duty in an adequate manner or:

- The disease manifests itself in excessive absenteeism; or
- The employee takes more than the standard amount of sick leave; or
- The employee displays irregular behaviour (e.g. AIDS dementia).

In the event that an employee eventually develops symptoms to the extent that he/she is unable to perform as required, the company's incapacity policy would then be implemented. Information on the company's

incapacity policy is available at/on/in *(where?)*_____. *(Insert where employees can access this information eg. HR, the intranet, each employee's employment contract).*

The company is obliged to provide a safe-working environment for all employees in line with the requirements laid down by prevailing legislation.

10. Confidentiality

Persons living with HIV/AIDS have the right to confidentiality and privacy concerning their health and HIV status. There should be no indicator on an employee's records if HIV status is known.

- All personal details of all employees, including the actual or suspected HIV status of any employee, shall remain strictly confidential.
- Any information about an employee's HIV status shall be revealed only with their written consent.
- An employee who contracts HIV will not be obligated to inform management. If an employee with HIV/AIDS decides to disclose his/her diagnosis to a colleague, superior or manager, the person will take all reasonable measures to ensure that this information remains private and confidential.
- The company will not tolerate any breaches in confidentiality. Any employee who breaches such confidentiality shall be subjected to appropriate disciplinary procedure. Speculation regarding another person's medical status will also not be tolerated.

11. Reasonable Accommodation

Employees who become unfit for work as a result of AIDS will be dealt with compassionately and in a just, humane and life-affirming way. The company will attempt to reasonably accommodate such individuals in less strenuous and stressful job, if possible. Failing this, the employee will be offered early retirement in accordance with standard company procedures and be entitled to appropriate benefits. Should early retirement not be applicable (i.e. the employee is not within 10 years of normal retirement age of 65) then the pension/provident fund benefit rules, as they pertain to permanent or temporary disability due to illness, will apply.

12. Colleagues of Employees who are HIV Positive

The company will ensure that effective educational programmes informing employees of the facts of HIV infection and AIDS are implemented. Should an employee, after reassurance and with all appropriate safety and health precautions being taken and supplied by the company, remain unwilling to work with an HIV positive employee, he/she will be warned that his/her reaction is unreasonable and scientifically unjust. He/she will be informed that his/her own employment situation is in jeopardy and that disciplinary action may be instituted.

13. Employees at Risk

The company will ensure that correct protective equipment is provided in all first aid boxes and that employees are trained in the correct use of this equipment.

The company will ensure that all first aid and healthcare workers are educated regarding HIV and AIDS infections; as well as other potentially infectious diseases; and that they understand and adhere to these standard operating procedures.

In addition to training on the facts of HIV/AIDS, the company will ensure that all employees will receive the following training to help them deal with HIV/AIDS in the workplace:

- How to deal with body fluids
- How to deal with stained linen
- How to deal with sanitary bins
- How to deal with used hypodermic needles
- How to deal with guest queries regarding HIV/AIDS and food
- How to deal with guest queries regarding HIV/AIDS in general
- Knowledge about HIV/AIDS and cutlery, crockery, food preparation, etc
- Knowledge about HIV/AIDS and sharing toilets
- How to deal with a colleague or guest who may be wounded, etc.

Where necessary the company will provide all relevant employees with appropriate protective clothing.

14. Policy Review

This policy will be reviewed and updated as and when the need arises or when developments relating to the above necessitate a review of the policy.

[illegible]

STUDY UNIT 5

Dealing with HIV in the workplace

LEARNING OUTCOMES

On completion of this study unit the student will....



- ♦ Understand the importance of writing a HIV/AIDS strategy.
- ♦ Explain and apply strategies to prevent HIV/AIDS in the workplace.
- ♦ Explain the term voluntary counseling.
- ♦ Explain how the workplace can provide support for HIV infected employees.
- ♦ Explain how employers can monitor and evaluate the HIV infected employees.
- ♦ Explain and apply monitoring and evaluation tools.

1. Introduction

Irrespective of your company's size, it's very important that you put your strategy in writing. That's because a written strategy will crystallize your thoughts, give you direction and clarify what you want to achieve.

What's more, when it comes to getting leadership/management buy-in, you'll need a written strategy to prove how serious you are about reducing the impact of HIV/AIDS on your company.



Remember, your strategy is **NOT** cast in stone. You can always revise and change it.

2. How to write your HIV/AIDS strategy

- Tick those sections which you want to include in your HIV/AIDS strategy. See attached table.
- Select a person/people who will take responsibility and be accountable for each action
- Clearly define each action
- Decide on a budget
- Set a realistic and suitable time plan
- Do it!
- Monitor and evaluate your strategy for overall effectiveness on an ongoing basis



Remember, you do not have to write a detailed strategy, or go to great effort to reinvent the wheel. The strategy table on pages 66—67 may well be good enough. If, however, you believe you need a more detailed strategy, use the table as a guide. Also remember that you shouldn't worry if you find that only one or two of the tools are appropriate for your business. After all, your strategy does have to be complex or difficult to implement.

Strategy	Implement? (tick if yes)	Specific Actions	Responsible person	Timeframe
Tools for the overall management of HIV/AIDS in your business				
Writing your HIV/AIDS strategy				
Ensuring leadership / and management buy in				
Appointing an HIV/ AIDS manager and organising a task team				
Ensuring compliance with the law				
Developing and implementing an HIV/AIDS workplace policy				
HIV/AIDS prevention tools				
Communicating your HIV/ AIDS policy				
Distributing condoms to employees				
Distributing condoms to guests/ clients				
Dealing with body fluids program				
Employee / guest communication training				
General hygiene and specific HIV/AIDS training				
Protective clothing for employees				
Safe first aid				
HIV/AIDS education and training				
First aid training				
Peer education				
Voluntary counseling and testing (VCT)				
STD treatment services				

Strategy	Implement? (tick if yes)	Specific Actions	Responsible person	Timeframe
HIV/AIDS treatment, care and support tools				
Public health service guidance				
Incapacity management				
Review of medical aid benefits				
Counseling				
Lifestyle and nutrition counseling				
Provision of balanced, nutritious meals				
Multivitamin supplements				
Private health care service				
Opportunistic infection treatment				
Anti-retroviral therapy				
Mother-to-child prevention				
Terminal care				
Monitoring and evaluation tools				
Organisational needs assessment				
Monitoring sick/compassionate leave				
Key performance indicators				
Organisational impact assessment				
Prevalence survey				
Impact on strategic markets/customers				
Impact on suppliers/ partners				
HIV/AIDS and your clients and community				
HIV/AIDS information for your clients				
Helping others in the industry				
Extension of your strategies to your surrounding community				

3. Obtaining management/leadership buy-in

Research shows that HIV/AIDS workplace programs are only successful if they are driven and supported by the leaders of the business i.e. the people who have the most influence.

If you're involved in a medium to super-large company, this tool will help you get the buy-in and support of your company's CEO, directors, managers etc as well as the buy-in of union officials and shop stewards.

If however you are the CEO or senior manager of a small company, you may have already obtained buy-in from your senior colleagues or partners, or the HIV/AIDS strategy decision may be yours alone. In this case, this tool is less relevant to you.

3.1 How to get management/leadership buy-in

To obtain buy-in from your company's management/leadership, use the following approach when presenting your HIV/AIDS strategy.

Step 1: Make them aware of your concerns about HIV/AIDS affecting the company

Step 2: Tell them about your plan of action and solutions i.e. what you propose doing

Step 3: Ask for their opinions and suggestions

Step 4: Get their consensus i.e. their agreement



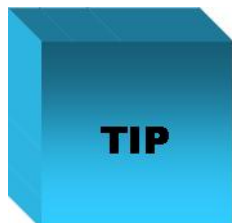
Remember, it's often easier to get buy-in from one person than from a group of people therefore try speak to the various people concerned on a one-to-one-basis.

3.2 Overcoming management/leadership blocks

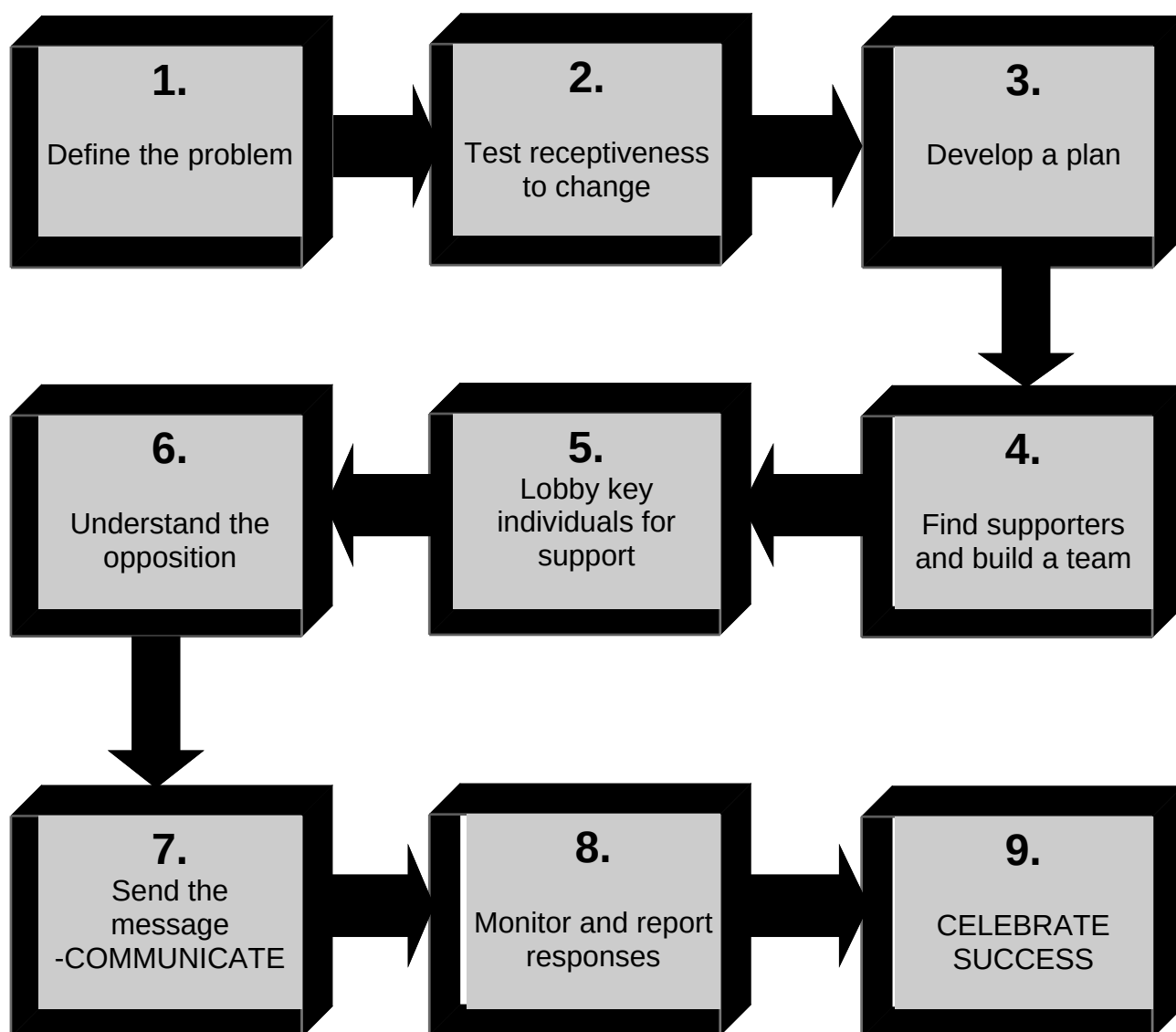
When presenting your strategy, don't get put off if people block you with any of the following negative responses:

- They deny the fact that HIV/AIDS is a business threat by saying things like "We don't have a problem".
- They hear what you're saying but are apathetic or don't want to take any action and so will say things like "The problem is too big for us, so leave it alone" or "Let someone else deal with it, not us".
- They block you with a prejudiced attitude towards people living with HIV/AIDS by saying things like, "It's not our fault they have HIV".
- They use the excuse of time pressures by saying things like, "We're far too busy to worry about HIV/AIDS issues".

- They use the excuse of not having budget, people and other resources needed to implement HIV/AIDS related activities by saying things like “We don't have any money or the people to implement anything in the workplace”.
- They use the excuse of not having sufficient information by saying things like “We don't know where to go to get support, so let's forget about it”, etc.



These kind of blocks are the main reason why workplace HIV/AIDS strategies get delayed or fail.



3.2 Action steps for overcoming management/leadership blocks

1. Make sure you clearly understand the block – define the problem.
2. Try to test the environment to assess management's receptiveness to change do this possibly by asking around to find out about management's overall attitude. Perhaps consult with a manager who you know well and confide in him/her to gather his/her perception on management's overall receptiveness to change.
3. You can now develop a plan to overcome management's block i.e. the best approach to convincing them to buy-in to your HIV/AIDS strategy. Your plan may include a presentation to management and key employees. To be convincing and drive the reality home, use statistics to support your case. For the latest HIV/AIDS statistics, refer to the resource lists on page 46 – 48 of this handbook.
4. If necessary, find supporters and build a team of employee representatives who will back you.
5. In addition, you may need to lobby support from key individuals i.e. labour or union representatives, key employees, etc.
6. If there are people opposed to your proposed plan, find out what makes them “tick” so that you are best equipped to deal with their negative comments.
7. At this point you can implement your plan – send your message. Remember that actions speak louder than words and try not to be over ambitious – start where you have some chance of success.
8. Don't forget to monitor and record responses to your actions.
9. If you still receive a negative response, don't despair go back to step one of the process.
10. When you do get management's buy-in, celebrate your success!

4. Appointing an HIV/AIDS manager and organising your task team

To ensure your HIV/AIDS strategy works and delivers results, you need to have an HIV/AIDS manager and/or a task team that will be responsible and accountable for making things happen.

REMEMBER.....

Remember, your task team needs specific goals, objectives and structure i.e. each and every one must be clear on what you want to achieve, and be given a role and responsibilities which they need to fulfill.

This section addresses these issues. Use it to choose the best person and/or people for the job, help you define your goals and objectives, and structure your team.

TAKE NOTE

If your company is small, you will just need an HIV/AIDS manager. However, if your company is medium, large or super-large, having an HIV/AIDS manager as well as a supportive task team will help you achieve more.

5. **Guidelines for forming and structuring an HIV/AIDS task team in medium, large and super large companies.**
 1. Based on your company's culture and size, think about how many people you ideally need to put your strategy into action. Remember, even if your company is large, you don't necessarily need to have a large team. Often a relatively small team will be more successful in making things happen.
 2. Look carefully at your company's structure in terms of divisions and departments, as well as your various employee levels, and then work towards building a team that is truly representative of your company. In other words, your team should comprise a mix of staff, male and female, ranging from senior and middle-level managers
 3. When you know the levels or divisions which need to be represented in your HIV/AIDS task team, you can do any of the following:
 - Send out a call for volunteers. That's not to say you should automatically bring all volunteers on board without first interviewing them. You should find out why they want to be involved and how they will play an active role in supporting and acting your strategy.
 - Have an election and let all employees nominate and vote for your task team members.
 - Ask management to identify the appropriate people.



Remember, representation from senior management is critical for success.

6. HIV/AIDS prevention in the workplace

There are a number of ways in which you can prevent the spread of HIV/AIDS in your company. These include:

- HIV/AIDS education and training
- Peer education
- Condom distribution
- Voluntary and testing (VCT)

This section gives you a number of tools that you can use. As you will see from the colour coding, most of these are “low cost and low effort”.

6.1 HIV/AIDS education and training

To ensure that your HIV negative employees remain negative, and those that are HIV positive, remain as well as possible and do not infect others, you need to make sure all your employees understand the important issues relating to HIV/AIDS.

To get the various messages across, you could run training sessions and/or use industrial theatre and/or printed educational material. Just ensure that your printed material is easy for all employees to understand. Check that the language used matches your employees' general literacy levels. If necessary, consider translating the material into appropriate languages.

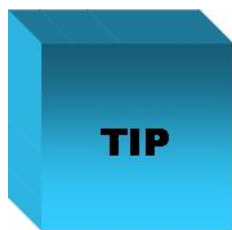
Remember, DO NOT exclude your company's management/leadership. They also need to know or be reminded of the basic facts. Plus they need to be equipped to deal with HIV/AIDS workplace issues.

What's more, management's participation in the training will prove just how seriously they take HIV/AIDS.

6.2 Content of your HIV/AIDS education and training sessions

Depending on your company's structure, size and culture, you could run different sessions as follows:

- General HIV/AIDS knowledge training for all employees covering:
 - ⇒ An explanation and description of HIV and AIDS
 - ⇒ How HIV is spread
 - ⇒ How your employees can prevent or reduce the risk of HIV infection
 - ⇒ Your employees' beliefs and attitudes about HIV/AIDS
 - ⇒ The human rights of people living with HIV/AIDS.



To establish the level of education required, first conduct an Employee Needs Assessment

- ◆ Specific industry HIV/AIDS knowledge training for appropriate employees covering:
 - ⇒ The potential risk of external clients not wanting to a site because of their concern about the country's HIV/AIDS statistics, plus how our high HIV/AIDS statistics might impact on them during their visit.
 - ⇒ The fact that clients often engage in more risky behaviour than when they are at home.
 - ⇒ How your employees should safely deal with injured clients and co-workers.
 - ⇒ The perceived risk of sharing equipment and toilets by an HIV positive person.



Remember, increased alcohol or drug use often leads to unprotected sex.

- ◆ Management training: To help your senior and middle level managers manage the impact of HIV/AIDS on your company, this session should include policy and management guidelines, as well as the legal issues relating to HIV/AIDS in the workplace.
- ◆ Industrial relations training: This session, targeted at your Industrial Relations/ Human Resources personnel should include information relating to HIV/AIDS workplace policy and legal issues.
- ◆ Union officials and shop steward training: This session should take the form of a presentation of your company's HIV/AIDS strategy. The reason being, you need union officials' and shop stewards' support and buy-in because they have a strong influence over your employees. In addition, they might need training on how to respond to employees' questions and queries.
- ◆ Support structure training: These sessions, targeted at your HIV/AIDS task team, healthcare workers and human resources, could include education and counseling training.

6.3 General HIV/AIDS information materials

Before you spend time and money on creating your own materials, contact the Department of Health about ordering their free brochures, leaflets and posters or download them from www.aidsinfo.co.za.

The Department of Health's materials include:

POSTERS

- ◆ Help prevent your baby from getting HIV
- ◆ Get tested for HIV
- ◆ Youth Campaign
 - ⇒ Event promotion
 - ⇒ I choose to be safe
 - ⇒ I choose to wait
 - ⇒ Not for sale
- ◆ Positive Living Nutrition
 - ⇒ Making healthy food
 - ⇒ Healthy eating with HIV
 - ⇒ Healthy living with HIV
- ◆ Sexually Transmitted Infections (STIs)
 - ⇒ Signs of STIs
 - ⇒ STIs can be cured
 - ⇒ Condoms
 - ⇒ Talk to your partner about STIs
- ◆ Tuberculosis
 - ⇒ Coughing for more than 2 weeks?

BOOKLETS:

- ◆ Teenagers and HIV
- ◆ Government Grants for children
- ◆ Keeping children safe and healthy at home
- ◆ Helping children deal with grief and death
- ◆ Keeping children in school
- ◆ Circles of support for our children
- ◆ Living positively with HIV and AIDS
- ◆ Talking about HIV and AIDS
- ◆ I had an HIV test – now what?
- ◆ Pregnancy and HIV – what you should now
- ◆ Stop TB because you can
- ◆ Sexual reproductive health

LEAFLETS

- ◆ Caring for people with HIV/AIDS
- ◆ Common sicknesses of people with HIV and AIDS
- ◆ Condoms
- ◆ HIV and AIDS Counseling
- ◆ HIV, AIDS and Relationships
- ◆ HIV, AIDS and Rights
- ◆ HIV and AIDS and the workplace
- ◆ Key points about HIV and AIDS
- ◆ Living with HIV and AIDS
- ◆ Sexually Transmitted Infections
- ◆ Stop Tuberculosis
- ◆ Why should I test for HIV?
- ◆ Take action to stop TB at work

CONTACT GUIDE

For information on HIV/AIDS educators and consultants, or if you need help with training and educational materials, please refer to the resource list at the end of this handbook.

6.4 Peer education

Often employees are more comfortable talking to their co-workers about HIV/AIDS related issues, and getting advice from them, rather than talking to their managers/supervisors.

Based on this, the idea of peer education is about using employees to educate, inform and advise their co-workers or peers on HIV/AIDS issues, provided they are correctly trained.

TAKE NOTE

Peer educators are generally volunteers.

Some guidelines for developing your peer education program

- ♦ Find peer educators employees who are willing to do the job in their free time, and have good communication skills.
- ♦ Ensure that your peer educators have a sound knowledge of HIV/AIDS, the rights of people living with HIV/AIDS, as well as your company's HIV/ AIDS workplace policy.
- ♦ When training your peer educators, discuss their indirect or direct experiences of HIV/AIDS as well as their attitudes towards HIV/AIDS and safe sex. Also use exercises such as role-playing and Questions & Answers.
- ♦ Give your peer educators all the printed educational material they need.
- ♦ Make sure your peer educators have a support network. Often, they too will need emotional support, especially when dealing with people in crisis.

6.4 Condom distribution amongst employees.

Remember, you can get free condoms, condom dispensers and leaflets on correct usage of condoms from the Department of Health. Female condoms are available at most pharmacies, however they are not free.

How to distribute condoms

Contact guide Safe, protected sex is about using condoms. They are vital for preventing the spread of HIV and all sexually transmitted infections (STIs) such as syphilis and herpes.

You can distribute condoms in any of the following ways:

- ♦ Install the dispensers in your employees 'toilet areas
- ♦ Remember to use the women's toilets as well as the men's.
- ♦ Install the dispensers in other public areas such as your staff canteen.
- ♦ If you use peer educators, they can distribute condoms.
- ♦ Insert condoms in employee pay packets.

6.5 Voluntary Counseling and Testing

Voluntary Counseling and Testing (VCT) which includes confidential HIV testing, together with pre- and post-test counseling is very important because:

- ♦ It is the only way in which your employees can know their status.
- ♦ It gives them an opportunity to think about their life style options and make appropriate choices.

7. HIV / AIDS treatment, care and support

Although there is no cure for HIV/AIDS, it can be treated. By making lifestyle changes and using correct medication, people living with HIV, can remain well and live for a longer time.

Treatment, care and support of your HIV positive employees, and possibly their families, can take various forms, this section gives you a number of different tools to consider. These include:

- Reviewing your company's medical aid benefits
- Helping employees access public health services
- Counseling
- Lifestyle and nutrition counseling and support
- On-site private healthcare services
- Post-exposure prophylactics
- Treatment of opportunistic infections
- Anti-retroviral therapy (ART)
- Incapacity management

7.1 Reviewing your company's medical aid benefits

If your company offers any/all employees medical aid cover, you need to know what, if any, HIV/AIDS benefits your medical aid offers, and the limits of these benefits.

Once you've checked and reviewed your medical aid's HIV/AIDS benefits, you may want to change your medical aid or negotiate increased benefit limits. Remember, generally speaking, the lower your employees' premiums, the lower their benefits limits. Therefore, they might not be adequately covered for comprehensive HIV/AIDS treatment and care.



REMEMBER

Remember, medical aid benefits and limits are not standardized, but vary from scheme to scheme. This is why reviewing your medical aid benefits is so important.



TAKE NOTE

Please note: Irrespective of your company's size, checking and reviewing your medical aid's HIV/AIDS benefits is very important.

7.2 Helping employees access public health services

If your company does not offer all employees medical aid, it's important that you give those who are not covered, information on the nearest public hospital or clinic where they provide HIV/AIDS treatment and care.

7.3 Counseling

Counseling helps both people who are infected with the HI Virus and those who are affected, deal with the various issues related to HIV/AIDS, including making plans for their families.

By providing counseling for your employees, you'll help alleviate their feelings of fear, anxiety, anger and depression - all of which impact on their work performance.

While it's preferable to use an external, professional counselor or a community service, you could consider training one or more employees in HIV/AIDS counseling. Please note, no employee should handle counseling unless they have received the correct training and understand the importance of confidentiality.

Counseling goals

The main goals of HIV/AIDS counseling include:

- ♦ To give information and psychological support to people who are HIV positive i.e.:
 - ⇒ come to terms with their status and help them work through their feelings
 - ⇒ make lifestyle choices and changes and plan for the future
- ♦ To give psychological support to the families of people who are HIV positive, and give them all the information on HIV/AIDS they need
- ♦ To ensure HIV positive people do not pass on the infection to their sexual partner (s) or unborn children
- ♦ To help HIV positive people stay as well as possible for as long as possible by controlling the HI Virus. For more information on this, refer to “**Lifestyle and nutrition counseling and support**”.

The role of the counselor

Counseling is a structured, confidential conversation in which the counselor:

- Educates, give information and provides guidance
- Is caring, compassionate and supportive
- Actively listens and communicates openly, honestly and directly
- Is not judgmental and respects the person's dignity, values and culture

Including the provision of counseling in your overall HIV/AIDS strategy

If you decide to offer your employees counseling, your strategy needs to define:

- ♦ The range of your counseling service
- ♦ Who can use it - will it be only for your employees, both those infected and affected? Or will it be extended to their families?
- ♦ Who will provide the counseling?
- ♦ How will you fund and sustain your counseling, etc.

7.4 Lifestyle and nutrition counseling and support

A healthy lifestyle which includes a balanced diet, rest, some exercise, as well as no smoking or alcohol, has been proven to keep HIV positive people well for longer and delay the onset of AIDS.

Lifestyle and nutrition counseling focuses on these issues. Besides talking to HIV positive people about what food they should eat and what lifestyle changes they need to make, counselors will also give HIV positive people appropriate eating plans and monitor their clients' progress and behaviour changes.

In addition to counseling, you could provide your HIV positive employees with meals and/or multivitamin supplements. Remember, there are specific multivitamins that have been proven to keep HIV positive people well for longer.

Including lifestyle and nutrition counseling and support in your overall HIV/AIDS strategy

If you choose to include any/all of these benefits in your strategy, you must define:

- ♦ Who qualifies for the benefit (s) -only HIV positive employees? Family members of HIV positive employees? Or all employees?
- ♦ How you will deliver the benefit (s)?
- ♦ How you will fund and sustain the benefit (s)?

7.5 On-site private healthcare services

If your company wants to provide employees (and their families) with an on-site private healthcare service, this could take the form of an on-site clinic. Alternatively, you could arrange for a doctor or nursing sister to visit your workplace on a weekly or monthly basis.

Including the use of on-site private healthcare services in your overall HIV/AIDS strategy

If you decide to use on-site private healthcare services, you need to include the detail in your strategy. For example:

- ♦ Who will provide the service?
- ♦ How the service will be funded?
- ♦ The frequency of the service, etc.

7.6 Post-exposure prophylactics

Prophylactics are drugs that prevent the development of HIV if someone has been exposed to the HI Virus.

Including the provision of post-exposure prophylactics in your overall HIV/AIDS strategy

If you decide to make post-exposure prophylactics available on-site or from your nearest clinic/hospital, your strategy should define the following:

- ♦ The circumstances under which you will provide post-exposure prophylactics e.g. rape
- ♦ Who qualifies for post-exposure prophylactics - all employees? Or family members of employees?
- ♦ Who will control and administrate your post-exposure prophylactics
- ♦ The treatment process
- ♦ How you will fund the cost, etc.

7.7 Treatment of opportunistic infections

People living with AIDS generally get various opportunistic infections (OIs) and cancers. By treating these infections promptly with correct medication, employees living with AIDS will have a better quality of life, be more functional and live longer.

Including the provision of OI treatment in your overall HIV/AIDS strategy

- ♦ The circumstances under which you will provide OI treatment \ who qualifies for OI treatment - all employees? Or family members of employees?
- ♦ Who will control and administrate the OIs?
- ♦ The treatment process
- ♦ How you will fund the cost, etc.

7.8 Anti-retroviral therapy

When anti-retroviral therapy (ART) is used correctly (i.e. at the right time with the right dosage), an HIV positive person will live longer, have a better quality of life and still be able to work productively. ART is therefore beneficial to both your HIV positive employees and your company.



REMEMBER

Remember, some medical aids offer some cover for ART. Check this when reviewing your medical aid's benefits.

Including the provision of ART in your overall HIV/AIDS strategy

If you decide to provide ART, your strategy should define:

- Who is entitled to ART - only your HIV positive employees? Or will their partners or family members who are HIV positive also receive them?
- When will ART be given - for information on this, speak to any of the recommended resources about CD4 counts, etc?
- How will you fund ART - will the company and employee share the cost?
- How will your ART program be managed and controlled?
- Who will check that employees are using their ART correctly?



Remember, ART cannot be shared. Please bear this in mind when defining who is entitled to this benefit. Also remember that anyone on ART needs to be counseled. They need to understand exactly how and when to take the drugs, and what dosage to take; and be made aware of the possible side-effects.

The difference between ART and ARV

Many people use these two terms interchangeably. However, ART refers to the entire treatment process, while ARV, which stands for Anti-retroviral, refers to a single drug.

7.9 Incapacity management

When an employee with HIV eventually develops symptoms that prevent him/her from fulfilling his/her job description, "incapacity" may be a valid reason to terminate their employment contract.

However, before you (the employer) use "incapacity" as a reason for employment termination, we recommend that you use the following as a guideline and that you use your own judgment when dealing with this difficult situation.

Guidelines for handling an employee's incapacity

- ♦ If an HIV positive employee's work performance and attendance record drops, he/she is entitled to the same amount of sick leave as any other employee. In terms of the Basic Conditions of Employment Act, standard sick leave is a minimum of 30 working days or equivalent to the number of days worked over 6 weeks, in a 3 year cycle.
- ♦ Once the employee has used all his/her standard sick leave, the employee can apply for additional unpaid leave. Whether you, the employer, say "yes" or "no" is your decision. However, you can achieve a win-win situation if the employee can work from home.
- ♦ If you grant the employee additional unpaid leave, you should first explain how having an absent employee, and how his/her incapacity to do their job 100%, impacts on your business. Then set a mutually agreed date for the employee's return to work.

- ♦ If, after the employee has returned to work, you find that he/she is still not meeting acceptable levels of performance or attendance, or has exceeded the sick leave allocation, you may ask the employee to give your permission to get a report from the employee's doctor or caregiver. Such a report should cover:
 - ⇒ How far the illness has progressed
 - ⇒ The prognosis
 - ⇒ The possible date on which the employee can resume his/her duties
 - ⇒ If you're unable to talk directly to the employee, you should then communicate in writing.
- ♦ The doctor will determine the content of the medical examination and will give you his/her opinion as to the employee's capacity to fulfill his/her job description. Remember, the doctor cannot by law give you a diagnosis of HIV/AIDS. That is strictly confidential information which only the employee can disclose.
- ♦ Employment termination should be your last option. Consider the feasibility and the practicality of the following options beforehand:
 - ⇒ Retaining the employee in his/her position
 - ⇒ Transferring the employee to a less demanding position - this may be in a totally different department, totally different job function, etc
 - ⇒ Looking into the possibility of working from home
 - ⇒ Obtaining the employee's agreement on a lower salary for less demanding work
 - ⇒ Making arrangements for flexi-time or part-time work
 - ⇒ Rearranging the job to share duties with others
 - ⇒ Arranging work that allows leave of absence where necessary, etc.
- ♦ before the employee goes on permanent or temporary disability, you need to ensure that he/she receives supportive counseling i.e. professional psychological counseling and an opportunity to understand everything and explain his/her feelings and point of view.
- ♦ If the employee is only temporarily unfit for work due to ill health, you should consider a temporary replacement.
- ♦ If, after you've had a full discussion with the employee, it is clear that he/she is unable to work, you may terminate his or her services in terms of the conditions of the employment contract, however you must follow the correct procedures.
- ♦ In some cases, employees who are declared temporarily or permanently unfit to work due to ill health, may qualify for benefits according to the company's pension/provident fund. If not, we encourage you to help the employee secure a UIF grant.

Including incapacity management in your overall HIV/AIDS strategy

Your strategy should include:

- ♦ A definition of "incapacity" as well as symptoms that would make the employee unable to work
- ♦ How your company will manage incapacity due to HIV/AIDS
- ♦ How incapacitated employees can access some form of income i.e. can they claim from your company's pension or provident fund?
- ♦ If your company does not have a pension/provident fund, you should include details on how your company will help incapacitated employees get a UIF grant (i.e. finding out about the nearest UIF office, how the money can be claimed, etc.)

8. HIV/AIDS monitoring and evaluation tools

Before actioning your HIV/AIDS strategy or any program, you first need to know what the issues of concern are. Then once you have identified these issues, and you action the appropriate program (s), you need to monitor or check how things are progressing.

Finally, once you have completed a program, you need to be able to evaluate or examine its impact and value.

EXAMPLE:

If you have run an HIV/AIDS education and training program, you'll need to know if more of your employees are using condoms, if they know more about HIV/AIDS and whether they have changed their attitudes towards people living with HIV/AIDS ETC.

What is monitoring?

DEFINITION

Monitoring involves checking the progress of a program or a strategy on a daily routine basis.

What is evaluation?

DEFINITION

Evaluation is about examining what you have achieved, and the impact of your program (s) or strategy.

REMEMBER

Remember, effective monitoring and evaluation is nothing more or less than an open and critically reflective communication process that will help you improve your HIV/AIDS strategy and activities, and strengthen relationships between people.

Why recording and reporting? Your senior management, shareholders and other stakeholders may well require information on your processes, programs and findings in writing. Be it for the purpose of ensuring good governance, legal compliance, your annual report or any other reason.

Even if they don't require this information, this information is important when planning or revising your HIV/AIDS strategy and workplace policy.

9. Types of monitoring and evaluation tools

There are a number of ways of monitoring and evaluating your HIV/AIDS strategy. These include:

- ♦ An organizational HIV/AIDS needs assessment
- ♦ An organizational impact assessment
- ♦ An organizational prevalence survey

This section gives you more information on these tools, all of which are colour-coded to give you an idea of the cost and effort involved.

TAKE NOTE

Please note: To check if your strategy is relevant and to evaluate the impact of your strategy, each of these assessments or surveys should ideally be repeated every 1 - 2 years. If this is not practical for your company, repeat the appropriate assessment/survey as often as possible.

9.1 Organisation HIV/AIDS needs assessments

An organizational HIV/AIDS needs assessment will help you identify your employee's needs regarding a range of HIV/AIDS issues. What it involves is developing a structured questionnaire for all employees to complete, analyzing the results and reporting on the findings.

TAKE NOTE

Please note: This assessment could be combined with an organizational HIV/AIDS impact assessment, HIV/AIDS prevalence testing and a KAP survey (These are described in this section).

Tip: Employees will feel more comfortable completing the questionnaire if they can do so anonymously.

9.2 Organisational HIV/AIDS impact assessments

This is a way in which you can assess the impact of HIV/AIDS on:

- Productivity
- Absenteeism
- Your medical aid and pension/provident fund premiums, etc

If you use this, you need to be clear about:

- ♦ What you expect from this assessment
- ♦ How it will be structured
- ♦ When it will be conducted, etc.

9.3 Organisational HIV/AIDS prevalence surveys

Organizational HIV/AIDS prevalence surveys give you a pretty good idea of **how many** of your employees are HIV positive, **not who**. With this knowledge, you can gauge the potential impact of HIV/AIDS on your company, and use the results when planning your company's short-, medium- and long-term strategy.

What does an HIV/AIDS prevalence survey involve?

An HIV/AIDS prevalence survey involves testing the saliva of at least 70% of all employees, across the board. Their names are not attached to their saliva samples, but the samples can be divided into age groups, male and female, or according to the structures/division of your company, etc.

Guidelines for implementing an HIV/AIDS prevalence survey

Due to the sensitive nature of this survey, it's imperative that you do it correctly and follow the correct procedures, to help you, use the following as a guide:

- ◆ Be clear about why you want to conduct a prevalence survey
- ◆ Know how you will segment your sample
- ◆ Know how the survey results will be used and to whom they will be communicated
- ◆ When you conduct the survey
- ◆ Use independent, external qualified consultants to handle and monitor the sampling, and give detailed reports with recommendations
- ◆ Consult the unions, get their buy-in and keep them informed about when you're doing the survey and the results
- ◆ Make the process totally transparent to all employees
- ◆ Get all employee's informed consent - remember, no-one can be forced to participate.



REMEMBER

Remember, to gain everyone's acceptance and co-operation, you need to make everyone aware of the benefits of conducting such a survey. People also need to be aware of the anonymity factor.

Understanding informed consent

Informed consent is about ensuring people having a complete understanding of a process e.g. a prevalence survey, before they agree to participate and sign any forms. If a person is illiterate or their English is poor, we recommend you get peer educators, counselors, your HR staff or co-workers to help them.

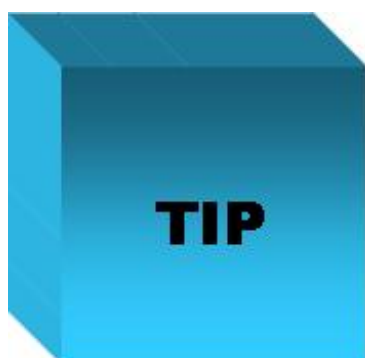
Benefits of HIV/AIDS prevalence surveys

The benefits of conducting an HIV/AIDS prevalence survey are:

- ♦ It gives you an idea of the number of HIV infections in your company and your company's future skills losses and shortages.
- ♦ Depending how you segment your sample, you can find where you have HIV infections i.e. in what divisions or levels of your company and/or age groups and/or gender groups you have any or a high infection rate.
- ♦ When you have these infection rates you can:
 - ⇒ plan for absenteeism, compassionate leave, retirement due to incapacity and recruitment and training of new staff
 - ⇒ work out the cost impact of HIV/AIDS on your company
 - ⇒ make fairly accurate projections regarding your company's short-and medium-term productivity, profitability, etc
 - ⇒ develop a comprehensive HIV/AIDS strategy with appropriate programs and interventions
- ♦ If you have already implemented any kind of HIV/AIDS prevention program, and HIV/AIDS prevalence survey will help you evaluate the effectiveness/impact of your program.

Prevalence testing amongst your target market and suppliers and partners

While this is a complicated process, knowing the prevalence of HIV/AIDS amongst your target market will give you an idea of the impact of HIV/AIDS on your revenue. Similarly, knowing the prevalence of HIV/AIDS amongst your suppliers and partners will give you an idea on their capacity and ability to continue delivering their services/products to your company.



To get employee and union buy-in to your prevalence survey, the benefit that is most relevant to them is your development of an HIV/AIDS strategy that prevents the spread of HIV/AIDS amongst employees, and includes appropriate forms of care and support.

WORDS/ACRONYMS	EXPLANATION OF ACRONYMS
ASSESSMENT	“A structured process of gathering evidence and making judgments about an individual’s performance in relation to registered national standards and qualifications”
ASSESSMENT CRITERIA	It describe the evidence that is needed to show that the learner has demonstrated the outcome correctly. A list of evidence to be supplied by the candidate as proof of his / her ability to integrate all of the competencies listed earlier in the standard.
COMPETENCE	Competence is the ability to do certain things in a range of different environments with knowledge and understanding about what is being done and the effect this has on others. It includes <u>Knowledge, skills, attitudes, values and ethics.</u>
CREDIT	Each unit standard is worth a number of credits which gives an indication of the size and scope of the unit standard, the standard is worth. (1 credit = 10 notional hours of learning)
CRITICAL CROSSFIELD OUTCOMES	critical cross field outcomes is to direct educational activities towards development of the learners within a social and economic environment.
EMBEDDED KNOWLEDGE	Body of theory essential for understanding and applying the standard. Principles, methods and techniques that would underpin good practice of this unit standard.
EVIDENCE	Evidence is a set of information or observations that provides proof of the learner’s competence.
FORMATIVE ASSESSMENT	Aims to assist the learner to identify any knowledge/skills gaps when demonstrating his/her current competence. Diagnostic assessment supports learning and progress of the learner.
INTEGRATED ASSESSMENT	Integrated assessment refers to: • assessing a number of outcomes together • assessing a number of assessment criteria together • assessing a number of unit standards together • using a combination of assessment methods and instruments for an outcome/s • collecting naturally occurring evidence in the workplace • Acquiring evidence from other sources such as supervisor’s reports, testimonials, portfolio of work previously done, logbooks, journals etc.
LEARNING ASSUMED TO BE IN PLACE	Building blocks (knowledge, skill and understanding) assumed to be in place
LEARNING PROGRAM	A “learning program is a set of learning activities in which the learner will be involved in working towards the achievement of one or more outcomes.
LEVEL	Each unit standard is registered on the NQF at a particular level based on the complexity of the outcomes and stringency of the standards.
NQF	National Qualifications Framework
RANGE STATEMENTS	The range statements indicate detailed requirements of the assessment criteria.

RE-ASSESSMENT	• The same method and instrument may be used, but, the task and materials should be different • The task and materials should, however, be of the same complexity and level as the previous ones. • In case the methods and instruments are changed it must be ensured that they are appropriate for the outcomes specified.
RPL ASSESSMENT	RPL is a way of recognizing what individuals already know and can do. The basic premise underlying it is that people, especially mature adults, learn many things inside and outside the formal structures of education and training and that irrespective of where, how and when learning was acquired, subject to assessment, it can be worthy of recognition and credit.
RPL	Recognition of prior learning
SASSETA	Safety & security sector education and training authority.
SAQA	SOUTH AFRICAN QUALIFICATION AUTHORITY
SPECIFIC OUTCOMES	An outcome is a statement that describes the required competency that must be demonstrated by the learner on successful completion of a training intervention. Key performance areas in the standard are given, the focus is <i>not</i> only on current competence, but also on future competence in this standard.
SUMMATIVE ASSESSMENT	Assessment used to record the level of competence achieved by a learner over a period of time, e.g. at the end of a program, term or year.
QUALIFICATIONS	Represent a planned combination of learning outcomes which has a defined purpose and intended to provide learners with applied competence and a basis for further learning
UNIT STANDARDS	the “building blocks” of qualifications. All qualifications are plotted on the National Qualifications Framework (NQF). “A statement of the outcomes that the learner must achieve to earn credit. Standards are nationally agreed and internationally comparable.”

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- ♦ <http://www.avert.org/aidssouthafrica.htm>
 - ♦ <http://www.engenderhealth.org/res/onc/hiv/understanding/hiv2p3.html>
 - ♦ <http://encyclopedia.thefreedictionary.com/aids>
 - ♦ The Foundation for AIDS Research ABOUT HIV/AIDS
 - ♦ SASETA HIV study (journal)

Apply knowledge of HIV/AIDS to a specific business sector and a workplace

SAQA US ID	UNIT STANDARD TITLE		
114941	Apply knowledge of HIV/AIDS to a specific business sector and a workplace		
ORIGINATOR		REGISTERING PROVIDER	
SGB Life Skills			
FIELD		SUBFIELD	
Field 07 - Human and Social Studies		People/Human-Centred Development	
ABET BAND	UNIT STANDARD TYPE	NQF LEVEL	CREDITS
Undefined	Regular-Fundamental	Level 3	4
REGISTRATION STATUS	REGISTRATION START DATE	REGISTRATION END DATE	SAQA DECISION NUMBER
Passed the End Date - Status was "Reregistered"	2007-06-26	2007-08-16	SAQA 0160/05
LAST DATE FOR ENROLMENT		LAST DATE FOR ACHIEVEMENT	
2008-08-16		2011-08-16	

PURPOSE OF THE UNIT STANDARD

This unit standard provides a broad introduction to HIV/AIDS in the workplace. It introduces knowledge about HIV/AIDS that will enable learners to be informed and caring workers in an industry and managers to develop appropriate policies to deal with the pandemic. The focus is knowledge, skills, values and attitudes in relation to the learner and management with a view to creating a caring environment in the workplace and the community.

A manager in the context of this Unit Standard is the manager of a small business and first line managers of business units, team leaders and supervisors in medium and large organisations. The term business unit implies a small business, cost centre, section or department.

The Unit Standard was developed as a means of contributing to change in leadership focus and empowering managers to look at people issues.

The qualifying learner is capable of:

- Explaining HIV/AIDS.
- Describing what behaviour is safe and what behaviour carries the risk of HIV/AIDS transmission.
- Interpreting data and trends on HIV/AIDS in order to explain the potential impact on a community, an organisation and a business sector.
- Investigating the guidelines and assistance that are available to support workers affected by HIV/AIDS.
- Explaining the implications of the HIV/AIDS pandemic for the community, the economy, an organisation and a specific workplace.

LEARNING ASSUMED TO BE IN PLACE AND RECOGNITION OF PRIOR LEARNING

It is assumed that learners are competent in Communication at NQF Level 1.

UNIT STANDARD RANGE

The typical scope of this unit standard is

- The effect of HIV/AIDS on the individual, the workplace and society.
- The effect of the HIV/AIDS pandemic on the economy, society, an organisation and a specific workplace.
- Transmission by body fluids and mother to child transmission.

UNIT STANDARD OUTCOME HEADER

N/A

Specific Outcomes and Assessment Criteria:

SPECIFIC OUTCOME 1

Explain HIV and Aids.

ASSESSMENT CRITERIA

ASSESSMENT CRITERION 1

The concepts HIV and AIDS are known and explained at a basic level of understanding.

ASSESSMENT CRITERION 2

The way in which the immune system works is explained with reference to the role of antibodies and how HIV attacks the immune system.

ASSESSMENT CRITERION 3

The concept of a window period is explained and an indication is given of how this affects the accuracy of test results.

ASSESSMENT CRITERION 4

The impact of opportunistic infections on the compromised immune system is explained with examples.

ASSESSMENT CRITERION 5

The stages of the disease are outlined with an example of what happens at each stage from infection with HIV to full-blown AIDS and death.

ASSESSMENT CRITERION 6

The ways in which HIV is transmitted are discussed with examples.

SPECIFIC OUTCOME 2

Describe what behaviour is safe and what behaviour carries the risk of HIV transmission.

ASSESSMENT CRITERIA

ASSESSMENT CRITERION 1

The chances of a person surviving with AIDS are discussed with reference to the latest views on wellbeing of an HIV positive person.

ASSESSMENT CRITERION 2

The relationship between human behaviour and HIV/AIDS is outlined and ways in which the individual can avoid contracting or spreading HIV/AIDS are indicated with reference to reducing the risk of infection.

ASSESSMENT CRITERION 3

Examples of behaviour that carry a risk of contracting HIV/AIDS are named and classified according to whether they carry a high, medium or low risk.

ASSESSMENT CRITERION 4

Situations that have a potential to spread HIV/AIDS in the workplace are discussed and rated in terms of high, medium and low risk.

ASSESSMENT CRITERION 5

Fears and common misunderstandings about the transmission of HIV are described in the context of the workplace.

SPECIFIC OUTCOME 3

Interpret data and trends in HIV/AIDS in order to explain the potential impact.

OUTCOME NOTES

Interpret data and trends in HIV/AIDS in order to explain the potential impact on an organisation or business sector.

ASSESSMENT CRITERIA**ASSESSMENT CRITERION 1**

Sources of data on HIV/AIDS are identified and appropriate statistics are accessed for a specific organisation.

ASSESSMENT CRITERION 2

Regional data is interpreted in order to identify trends.

ASSESSMENT CRITERION 3

Regional data is applied to a specific sector or industry.

ASSESSMENT CRITERION 4

The potential impact of HIV/AIDS on an organisation is inferred from regional and sector data.

ASSESSMENT CRITERION RANGE

Includes, but is not limited, to revenue generation, absenteeism, productivity, Human Resource policies and programmes, employment practices, mechanisation, staff turnover and cost of health care.

SPECIFIC OUTCOME 4

Investigate the guidelines & assistance that are available to support workers affected by HIV/AIDS.

ASSESSMENT CRITERIA**ASSESSMENT CRITERION 1**

A company policy on HIV/AIDS or the National Department of Health's document Guidelines for developing a workplace policy and programme is presented graphically.

ASSESSMENT CRITERION 2

The possible problems that a worker with HIV/AIDS could encounter are listed with suggestions as to what the learner could do to create a caring situation in the workplace.

ASSESSMENT CRITERION 3

The importance of employers playing a proactive role in addressing the AIDS pandemic are explained and suggestions are made as to how a specific organisation could create a caring environment for workers with HIV/AIDS.

ASSESSMENT CRITERION 4

The availability of HIV/AIDS prevention and wellness programmes is known and an indication is given of how to access these in a specific organisation or area.

ASSESSMENT CRITERION 5

Legislation that relates to HIV/AIDS in the workplace is identified and an indication is given of the consequences of non-compliance.

ASSESSMENT CRITERION RANGE

Includes, but is not limited to, LRA, BCEA, SA Constitution, OHSA, EE, Health and Hygiene and industry specific legislation.

ASSESSMENT CRITERION 6

The Universal Precautions are known and applied in the work environment.

ASSESSMENT CRITERION 7

The concept of Voluntary Counselling and Testing (VCT) is explained with reference to the role of two different tests.

ASSESSMENT CRITERION 8

The effects of HIV infection are explained including the ability of infected persons to perform work and the importance of lifestyle changes to boost the immune system and prevent re-infection.

ASSESSMENT CRITERION 9

The importance of pre and post test counselling is known and understood and the implications of HIV testing for an individual are discussed in terms of making a personal decision to take an AIDS test.

SPECIFIC OUTCOME 5

Explain the implications of the HIV/AIDS pandemic.

OUTCOME NOTES

Explain the implications of the HIV/AIDS pandemic for the community, the economy, an organisation and a specific workplace.

ASSESSMENT CRITERIA**ASSESSMENT CRITERION 1**

The effect of a population composed mainly of children and the aged on the economy and the State is outlined with examples.

ASSESSMENT CRITERION 2

The need for medical care for people with HIV/AIDS and the implications for employers and the State are discussed with examples.

ASSESSMENT CRITERION 3

The effect of HIV/AIDS on the workforce and family income is discussed with examples.

ASSESSMENT CRITERION 4

The implications of HIV/AIDS for an organisation and a specific workplace are discussed with reference to revenue generation, absenteeism, productivity, Human Resource policies and programmes, employment practices, mechanisation, staff turnover and cost of health care.

UNIT STANDARD ACCREDITATION AND MODERATION OPTIONS

This Unit Standard will be internally assessed by the provider and moderated by a moderator registered by a relevant accredited ETQA or an ETQA that has a Memorandum of Understanding with the relevant accredited ETQA.

UNIT STANDARD DEVELOPMENTAL OUTCOME

N/A